Answer Key & Rationales · Form 5

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.22-0834.

#	Key	Area	Sub-area	P
1	**B**	I	I.B	
2	**A**	II	II.B	
3	**C**	II	II.B	
4	**B**	II	II.B	
5	**A**	III	III.B	
6	**B**	III	III.A	
7	**D**	III	III.B	
8	**B**	I	I.B	
9	**B**	II	II.A	
10	**A**	III	III.B	
11	**B**	III	III.A	
12	**C**	II	II.A	**P**
13	**A**	I	I.C	
14	**A**	III	III.B	
15	**A**	I	I.C	
16	**B**	II	II.C	
17	**D**	I	I.B	
18	**B**	III	III.B	**P**
19	**C**	I	I.B	
20	**C**	III	III.B	
21	**A**	II	II.B	
22	**B**	I	I.A	
23	**D**	I	I.A	
24	**A**	III	III.A	
25	**B**	III	III.B	
26	**A**	III	III.B	
27	**D**	III	III.B	
28	**C**	I	I.A	
29	**A**	III	III.B	**P**
30	**A**	I	I.B	
31	**C**	II	II.A	
32	**A**	I	I.A	
33	**B**	II	II.C	
34	**B**	III	III.B	
35	**B**	II	II.A	
36	**C**	II	II.B	
37	**C**	I	I.A	
38	**A**	II	II.A	
39	**B**	II	II.C	
40	**C**	II	II.B	
41	**B**	II	II.A	
42	**B**	II	II.C	**P**
43	**A**	I	I.B	
44	**D**	I	I.B	
45	**C**	I	I.B	
46	**C**	I	I.A	**P**
47	**C**	III	III.C	
48	**A**	I	I.C	
49	**A**	III	III.D	
50	**B**	III	III.B	
51	**B**	I	I.A	
52	**D**	II	II.A	
53	**C**	III	III.B	**P**
54	**B**	I	I.C	
55	**D**	I	I.C	
56	**B**	III	III.C	
57	**B**	II	II.A	
58	**B**	I	I.B	
59	**A**	III	III.D	
60	**B**	II	II.A	
61	**C**	II	II.A	
62	**D**	I	I.B	
63	**D**	II	II.B	
64	**B**	III	III.B	
65	**B**	I	I.A	
66	**C**	I	I.B	
67	**C**	I	I.A	
68	**A**	II	II.B	
69	**B**	I	I.B	
70	**C**	II	II.B	
71	**C**	I	I.B	
72	**A**	I	I.A	
73	**A**	III	III.D	
74	**A**	III	III.B	
75	**D**	II	II.C	
76	**C**	II	II.A	
77	**A**	II	II.B	**P**
78	**A**	III	III.B	**P**
79	**C**	II	II.A	
80	**A**	III	III.B	
81	**C**	II	II.B	
82	**D**	II	II.B	
83	**D**	I	I.A	
84	**A**	I	I.B	
85	**C**	III	III.B	
86	**C**	III	III.B	
87	**B**	I	I.C	
88	**C**	III	III.C	
89	**A**	III	III.D	
90	**B**	III	III.A	
91	**C**	III	III.C	
92	**B**	II	II.A	
93	**A**	III	III.B	
94	**C**	II	II.A	
95	**C**	III	III.C	
96	**A**	I	I.B	
97	**D**	I	I.C	
98	**B**	II	II.A	
99	**D**	I	I.B	
100	**B**	III	III.A	
101	**C**	I	I.C	
102	**A**	II	II.C	
103	**A**	II	II.B	**P**
104	**C**	II	II.A	
105	**A**	I	I.A	
106	**A**	II	II.A	
107	**C**	I	I.C	
108	**B**	III	III.B	
109	**A**	I	I.B	**P**
110	**C**	II	II.A	
111	**C**	I	I.A	**P**
112	**A**	III	III.B	
113	**D**	I	I.C	
114	**A**	II	II.A	
115	**A**	I	I.A	**P**
116	**C**	I	I.A	
117	**C**	III	III.A	
118	**B**	I	I.B	
119	**C**	III	III.B	
120	**B**	I	I.C	
121	**A**	II	II.B	
122	**A**	III	III.B	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**___ / 40**
I.A Ethical Principles and Responsibilities	22, 23, 28, 32, 37, 51, 65, 67, 72, 83, 105, 116	12	____
I.B Ethical Service Delivery	1, 8, 17, 19, 30, 43, 44, 45, 58, 62, 66, 69, 71, 84, 96, 99, 118	17	____
I.C Diversity and Social Justice	13, 15, 48, 54, 55, 87, 97, 101, 107, 113, 120	11	____
II. Assessment and Planning		**35**	**___ / 35**
II.A Assessment Concepts	9, 31, 35, 38, 41, 52, 57, 60, 61, 76, 79, 92, 94, 98, 104, 106, 110, 114	18	____
II.B Assessment Methods and Techniques	2, 3, 4, 21, 36, 40, 63, 68, 70, 81, 82, 121	12	____
II.C Assessment Practices	16, 33, 39, 75, 102	5	____
III. Intervention and Practice		**35**	**___ / 35**
III.A Practice Concepts	6, 11, 24, 90, 100, 117	6	____
III.B Intervention Methods and Techniques	5, 7, 10, 14, 20, 25, 26, 27, 34, 50, 64, 74, 80, 85, 86, 93, 108, 112, 119, 122	20	____
III.C Practice Evaluation and Research	47, 56, 88, 91, 95	5	____
III.D Supervision and Administration	49, 59, 73, 89	4	____

Pretest items (skip when scoring): 12, 18, 29, 42, 46, 53, 77, 78, 103, 109, 111, 115

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. ETH-1060 (rev 1) · Best answer: B

At 10 p.m., a social worker scrolling a public community page sees a post made 15 minutes earlier by a current adult client. It shows a pile of pill bottles with the words 'Done fighting. Goodbye.' The social worker calls the client's phone and reaches voicemail. What should the social worker do **NEXT**?

A. Text the client to ask about current safety
B. Request an emergency welfare check at the client's home
C. Message the client on the platform asking for a call back
D. Discuss at the next session how the post was found

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Asking about safety is right when risk is unclear. Here a goodbye message with pills and an unanswered call already establish imminent danger, and waiting on a text reply loses critical time.

B **Correct.** Consent is normally required before gathering client information online, but an exception exists to protect a client from serious, imminent harm (NASW Code 1.03(i)), and disclosure to prevent such harm is permitted (1.07(c)). A goodbye message with pills, posted minutes ago, plus an unanswered call, shows imminent risk. B beats A because a text, like the call, may go unanswered while time passes; once direct contact has failed, emergency responders need to check on the client. The worker can be open with the client about the post afterward.

C **Incorrect** (trap: Misuses the technique). Reaching out through the social media platform mixes personal channels with clinical contact and, like the call, may go unanswered. Imminent risk needs emergency responders, not another message.

D **Incorrect** (trap: Right action, wrong time). Being open with the client about how the post was found is appropriate, but only after the client is safe. Waiting until the next session ignores imminent risk.

Takeaway: Online information found without consent may be acted on to prevent imminent harm; when direct contact fails, send emergency help.

Sources: NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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2. ASM-1580 (rev 1) · Best answer: A

On a home visit, the social worker explains how a client can book rides to dialysis through a new transportation service. The client nods throughout and says, 'Yes, I've got it.' Two ride requests the client tried to make last month were never completed. Which response is **MOST** appropriate?

A. Ask the client to show how they would book the next ride
B. Ask the client whether they have any questions about it
C. Leave a written copy of the booking steps with the client

Why each option is right or wrong

A **Correct.** Teach-back checks understanding by having the person explain or demonstrate the information in their own words, because people often say they understand when they do not (AHRQ, Teach-Back). The client's nodding contrasts with two failed bookings, so the worker needs evidence of understanding, not agreement. Teach-back also shows whether the explanation itself was clear. It beats C because a handout may help later, but it cannot reveal which step is confusing the client.

B **Incorrect** (trap: Misuses the technique). Asking for questions is closed and puts the burden on the client to notice and admit confusion. Many people answer no out of embarrassment or because they believe they understand.

C **Incorrect** (trap: Good but not best). Written steps are a useful reminder, but they assume the client can read and follow them, and they do not show whether the client understood. Give them after teach-back confirms understanding.

Takeaway: Check a client's understanding with teach-back, asking them to explain or demonstrate, rather than accepting nods or 'Any questions?'

Sources: Agency for Healthcare Research and Quality (AHRQ), TeamSTEPPS Curriculum, Communication Tools: Teach-Back

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.8 Evaluating a client's communication skills · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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3. ASM-1675 (rev 2) · Best answer: C

A cardiac rehabilitation team describes a 74-year-old patient as 'unmotivated' because the patient rarely does the home exercises. The patient says they want to get stronger. The social worker notices that the patient asks the same questions several times per visit and got lost driving to the clinic last week. What should the social worker do **FIRST**?

A. Use motivational interviewing to build commitment to exercising
B. Give the patient a simplified written home exercise schedule
C. Ask the physician to evaluate the patient's memory problems
D. Ask a family member to supervise the daily exercises

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Motivational interviewing fits ambivalence about change, but this patient already voices a wish to get stronger. The missed exercises track with memory and orientation problems, so building commitment would target the wrong barrier (Miller & Rollnick, 2023).

B **Incorrect** (trap: Right action, wrong time). Simplified written instructions are a sensible support for someone with memory problems and may become part of the plan. Offered first, they skip the evaluation that could find a treatable cause of the decline.

C **Correct.** Assess barriers before concluding that a client lacks motivation. This patient states a wish to improve, yet repeats the same questions within a visit and got lost driving to the clinic. These signs point to a possible cognitive barrier rather than ambivalence. Observed lapses do not establish a cause, and some causes of cognitive decline are treatable, so a medical evaluation comes first (Kaplan & Sadock's Synopsis). It beats B because simplified written steps help most once the team knows

what kind of impairment is present; they can support the plan but cannot replace the evaluation.

D **Incorrect** (trap: Good but not best). Family help can support home exercise, but it treats the problem as follow-through alone. It leaves the memory and orientation problems unevaluated and may take over a task the patient wants to manage.

Takeaway: When a client wants change but does not follow through, assess for cognitive or other barriers before labeling the client unmotivated.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Neurocognitive disorders: evaluation of cognitive impairment and reversible causes; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Ambivalence and the spirit of MI

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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4. ASM-1583 (rev 2) · Best answer: B

At an annual planning meeting in a day program, a 40-year-old client with intellectual developmental disorder sits beside a parent who is the client's legal guardian. Each time the social worker asks about goals for the coming year, the parent answers for the client. The client speaks in short sentences and understands pictures well. Which action is **MOST** appropriate?

A. Record the parent's answers as the client's goals for the year

B. Ask the client directly, using plain words and picture choices

C. Ask the parent to leave so the client can speak more freely

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). The guardian's views carry legal weight, but recording them as the client's goals skips the client's own voice. Guardianship does not remove a person's right to be heard about their life and goals.

B **Correct.** Clients take part in planning to the greatest extent they can, and when a substitute decision maker is involved, social workers still inform clients at their level of understanding and safeguard their interests and rights (NASW Code 1.03(c), 1.14). The client communicates in short sentences and understands pictures, so adapting the questions lets the client state goals while the guardian stays involved. It beats C because removing the guardian sidelines a person with legal authority over decisions, when adapting the conversation achieves the same aim (NASW Code 1.02).

C **Incorrect** (trap: Misreads law or ethics). Time alone can help some clients speak, but excluding the legal guardian from a formal planning meeting disregards the guardian's role. Adapting communication with the guardian present respects both people's roles.

Takeaway: When a guardian speaks for a client, adapt communication so the client can state their own goals while the guardian stays involved.

Sources: NASW Code of Ethics (2021), 1.14; NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(c)

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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5. INT-1086 (rev 1) · Best answer: A

During a video session, a client describing a carjacking stops mid-sentence, stares past the camera, and does not respond when the social worker says the client's name. After a long pause, the client whispers, 'I'm not really here.' What should the social worker do **FIRST**?

A. Help the client notice objects and sounds in the room
B. Ask the client what feelings are coming up about the carjacking
C. Encourage the client to keep going and finish the story
D. End the session early so the client can rest

Why each option is right or wrong

A **Correct.** When a client dissociates, the worker pauses trauma content and uses grounding to bring attention back to the present, such as naming things the client can see, hear or touch (SAMHSA TIP 57, 2014). Staring, not responding to their name and saying 'I'm not really here' show the client has left the window of tolerance (Siegel, 1999). It beats B because asking about feelings keeps the focus on the traumatic memory and needs a level of presence the client lacks right now; feelings can be explored once the client is oriented.

B **Incorrect** (trap: Right action, wrong time). Exploring feelings is useful when the client is present and within tolerance. Right now the client is dissociated, and turning back to the carjacking deepens the disconnection. Grounding must come first.

C **Incorrect** (trap: Ignores risk). Finishing the story while dissociated does not process the memory and can reinforce overwhelm. The narrative can resume, if appropriate, once the client is grounded and able to stay present.

D **Incorrect** (trap: Ignores risk). Ending the call leaves a dissociated client alone, which is unsafe on video where the worker cannot help in person. The client should be grounded and oriented before the session closes, with a plan for the rest of the day.

Takeaway: When a client dissociates during trauma work, pause the content and use grounding to reorient them to the present before anything else.

Sources: SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment (Exhibit 1.4-1, Grounding Techniques); Siegel, D. J. (1999). The Developing Mind. Guilford Press, window of tolerance
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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6. INT-1559 (rev 1) · Best answer: B

A child welfare social worker supports a parent whose 3-year-old is in foster care. The parent completed a 10-week parenting class and earned a certificate. During supervised visits, the parent gives sharp commands and rarely praises, and the child cries and pulls away. The case plan review is in 6 weeks, and the parent wants to reunify. Which action is **BEST** before the review?

A. Enroll the parent in a second class on parenting young children
B. Coach the parent in real time during their play together
C. Record the class certificate as meeting the parenting goal

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A second course adds knowledge but repeats the format that left skills unchanged at visits. It might help if the parent lacked basic information; here the gap is applying skills with the child.

B **Correct.** Parent training works best when parents practice new skills with their own child and receive feedback, a component linked to larger effects in a meta-analysis of parent training programs (Kaminski et al., 2008). The certificate shows attendance, but the visits show the skills have not carried over. Live coaching targets praise and calm commands while the child is present. It beats A because more classroom teaching repeats the format that has not yet changed the parent's behavior with this child.

C **Incorrect** (trap: Misreads law or ethics). Completion is not the same as change. Recording the goal as met when visits show harsh commands and a distressed child misrepresents progress to the court and team; the record should reflect observed behavior.

Takeaway: Judge parenting progress by observed skills with the child, and build skills through practice with live feedback, not certificates alone.

Sources: Kaminski, J. W., Valle, L. A., Filene, J. H., & Boyle, C. L. (2008). A meta-analytic review of components associated with parent training program effectiveness. Journal of Abnormal Child Psychology, 36(4), 567-589, whole article

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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7. INT-1130 (rev 1) · Best answer: D

In the third week of an 8-week MBSR group, a member with PTSD grips the chair and breathes rapidly during the body scan. Afterward, the member says the exercise brought back the assault and the room felt far away. The member wants to stay in the group. The social worker is trained in MBSR. What is the **BEST** response?

A. Encourage the member to keep attention on body sensations as they arise
B. Help the member process the assault memory in a private session
C. Arrange individual trauma therapy in place of the group sessions
D. Offer options such as eyes open and an outside focus during practice

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Turning toward sensations is the standard MBSR instruction, and it helps many members. For a member who is dissociating, more inward body focus can deepen the flashback. Trauma-sensitive practice shifts attention to steadier anchors first.

B **Incorrect** (trap: Good but not best). Processing the memory is trauma treatment, not part of a mindfulness group, and it does not address what the member should do during the next body scan. Memory work belongs in a planned trauma therapy if the member chooses one.

C **Incorrect** (trap: Hands off too soon). A trauma therapy referral can be offered as an addition if the member wants it. Making it a replacement overrides the member's wish to stay and skips simple changes that often make practice tolerable.

D **Correct.** Trauma-sensitive mindfulness keeps practice within the survivor's window of tolerance by offering choices, such as eyes open, shorter periods, or anchoring attention on sounds or the feet instead of inner sensations (Treleaven, 2018). The gripping, rapid breathing, and sense of distance signal hyperarousal and dissociation, yet the member wants to continue. Adjusting the practice honors that choice, a core trauma-informed principle (SAMHSA, 2014), while lowering risk. It beats C because removing the member from a valued group is not needed when simple changes can make practice safe.

Takeaway: When mindfulness practice triggers trauma symptoms, adapt it with choices and external anchors before removing the client from it.

Sources: Treleaven, D. A. (2018). Trauma-Sensitive Mindfulness: Practices for Safe and Transformative Healing. W. W. Norton, window of tolerance; shifting attention to stabilizing anchors; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: empowerment, voice and choice
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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8. ETH-1056 (rev 1) · Best answer: B

A social worker in a home-based family program carries a large caseload. The agency's electronic record lets staff copy the previous visit's note into a new note. A colleague suggests copying notes forward to keep up with documentation. How should the social worker use this feature?

A. Copy each note forward and update the visit date
B. Edit each copied note to reflect that visit
C. Write one summary note every two weeks instead

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Changing only the date makes each note claim that the earlier visit's events happened again. Identical cloned notes misrepresent services and draw audit scrutiny, even when the visits occurred.

B **Correct.** Records must accurately reflect the services provided and be timely enough to support continuity (NASW Code 3.04(a), 3.04(b)). Copy-forward can serve as a starting draft, but each note must then be revised to describe what happened at that visit: who attended, what was done and how the family responded. B beats A because a note copied with only a new date repeats old content as if it recurred, which misrepresents services and can support inaccurate billing (3.05).

C **Incorrect** (trap: Misreads law or ethics). A summary every two weeks fails the timeliness standard (3.04(b)). Each visit needs its own note, written promptly, so that others can rely on the record.

Takeaway: Electronic shortcuts are acceptable only if every note is edited to reflect the service actually delivered.

Sources: NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.05
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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9. ASM-1096 (rev 1) · Best answer: B

A home-delivered meals worker refers a 78-year-old client with mild memory problems. A new acquaintance moved into the client's home 2 months ago and now collects the mail. The electricity is about to be shut off despite a steady pension, and the bank has called about large ATM withdrawals. The client says the acquaintance 'handles everything.' What form of mistreatment is **MOST** likely occurring?

A. Self-neglect
B. Financial exploitation
C. Caregiver neglect
D. Psychological abuse

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Self-neglect could explain unpaid bills in a person with memory problems. It does not explain large withdrawals and a new housemate taking over the client's mail and affairs.

B **Correct.** Financial exploitation is the improper use of an older adult's money or property. Red flags include sudden unexplained bank activity such as large withdrawals, bills left unpaid despite adequate resources, and someone new taking control of finances (U.S. Department of Justice, Elder Justice Initiative). A new housemate who collects the mail while withdrawals rise and utilities lapse fits this pattern. It beats A because self-neglect could explain unpaid bills but not the withdrawals and the new person managing the client's affairs.

C **Incorrect** (trap: Near-miss diagnosis). Caregiver neglect is failure to meet basic needs such as food, hygiene or medical care. The facts here center on money moving out of the account, not on unmet physical care.

D **Incorrect** (trap: Near-miss diagnosis). Psychological abuse involves threats, humiliation or isolation. Control by the housemate may involve some of this, but the clearest pattern is the misuse of the client's money.

Takeaway: Unexplained withdrawals, unpaid bills despite adequate income and a new person controlling finances signal elder financial exploitation.

Sources: U.S. Department of Justice, Elder Justice Initiative (2023). Red Flags of Elder Abuse, Financial Exploitation
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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10. INT-1551 (rev 2) · Best answer: A

A hospital emergency department social worker refers patients treated for opioid overdose to a community clinic that offers medication for opioid use disorder. For 2 years, a clinic nurse personally scheduled each referred patient. Since that nurse left 3 months ago, 7 of 9 referred patients never received an intake. Clinic leaders say they want the partnership to work. What is the social worker's **BEST** response?

A. Develop a written referral agreement with clinic leadership
B. Find a new staff member at the clinic to schedule intakes
C. Phone the clinic to book each patient's intake before discharge

Why each option is right or wrong

A **Correct.** Service networks last when the links between organizations are built into structure rather than resting on one person; NASW's case management standards note that written interorganizational agreements can be useful (NASW Standard 8). The pathway collapsed when the nurse left, and clinic leaders are willing partners. A written agreement can name roles, a scheduling process and a feedback loop that survive staff turnover. It beats B because a new personal contact restores referrals only until that person leaves, repeating the same failure.

B **Incorrect** (trap: Good but not best). A new contact may restore referrals quickly and could be named in a formal agreement. Relying on another single relationship leaves the pathway as fragile as before, which is exactly what failed when the nurse left.

C **Incorrect** (trap: Good but not best). Booking intakes from the emergency department may restore referrals quickly and is a good stopgap. It moves the single point of failure from the clinic nurse to the social worker, though, so the pathway still rests on one person's effort rather than an agreed process between the two organizations.

Takeaway: Make interagency referral pathways durable through formal agreements with defined roles, not one staff member's personal relationships.

Sources: NASW Standards for Social Work Case Management (2013), Standard 8: Interdisciplinary and Interorganizational Collaboration

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.24 Building service networks and community resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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11. INT-0100 (rev 1) · Best answer: B

By the fourth session, a social worker notices growing impatience each time a client describes relying on relatives for help. The social worker recognizes that the reaction echoes long-standing feelings about a dependent family member in the social worker's own life. What should the social worker do **NEXT**?

A. Tell the client about the reaction to keep the relationship genuine

B. Explore the reaction with a supervisor or consultant

C. Transfer the client to a colleague to protect the alliance

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Disclosing countertransference shifts the focus to the social worker's feelings and can burden or confuse the client. Self-disclosure should serve the client's needs, and the reaction needs to be understood in supervision first.

B **Correct.** Countertransference is the social worker's own emotional reaction to a client, often rooted in the worker's history. Once it is recognized, the next step is to examine it in supervision or consultation so it informs, rather than drives, the work (Gelso & Hayes, 2007; NASW Code 2.05(a)). Here the worker has already named the source of the impatience, so exploring it with a supervisor protects the client from subtle withdrawal or judgment. It beats C because a transfer ends a relationship that is not yet harmed, before the worker has tried to understand and manage the reaction.

C **Incorrect** (trap: Hands off too soon). Transfer may become appropriate if supervision shows the reaction cannot be managed and is harming the work. As a next step, it hands off a client the worker has not yet tried to serve with support.

Takeaway: When countertransference is recognized, examine it in supervision or consultation before considering disclosure or transfer.

Sources: Gelso, C. J., & Hayes, J. A. (2007). Countertransference and the Therapist's Inner Experience: Perils and Possibilities. Lawrence Erlbaum, managing countertransference; NASW Code of Ethics (2021), 2.05(a)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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12. ASM-1181 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

A hospital social worker is asked to meet with the parent of a 6-year-old whose lab results show gonorrhea. The physician confirms the infection could not have been passed at birth. The parent says the child must have caught it from shared towels at a relative's home. What should the social worker do **FIRST**?

A. Record the towel explanation and arrange the child's treatment
B. Arrange a forensic interview at a children's advocacy center
C. Notify the child abuse hotline of the possible sexual abuse

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Treatment follow-up is needed, and the parent's account belongs in the record. But shared towels do not credibly explain gonorrhea in a child; past the newborn period it strongly indicates sexual abuse (CDC, 2021). Accepting the explanation leaves the child unprotected.

B **Incorrect** (trap: Right action, wrong time). A forensic interview is the right way to hear from the child, but it is arranged through the investigation the report sets in motion. Arranging it first delays the mandated report.

C **Correct.** An STI such as gonorrhea in a child past the newborn period strongly indicates sexual abuse (CDC, 2021). That meets the reasonable-suspicion standard behind mandated reporting laws (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)). The towel explanation does not remove the duty, because reporters do not need proof and do not judge whether abuse occurred. It beats B because the forensic interview is arranged through the investigation that the report starts.

Takeaway: Gonorrhea or another STI acquired after birth in a young child signals likely sexual abuse and warrants a report, whatever caregivers say.

Sources: CDC (2021). Sexually Transmitted Infections Treatment Guidelines, Sexual Assault or Abuse of Children; Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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13. ETH-1680 (rev 1) · Best answer: A

A social worker who manages an outpatient program reviews caseloads and finds that the program's two Black staff social workers carry nearly all of the Black clients. They are also asked to lead every diversity training, with no change in workload. One of them recently said, 'I'm running on empty.' What should the manager do **FIRST**?

A. Meet with the two social workers about the pattern and their workload
B. Spread new Black clients evenly across the whole staff
C. Add a stipend for the extra diversity training workload
D. Suggest the tired staff member use the employee assistance program

Why each option is right or wrong

A **Correct.** Social workers act to prevent discrimination in work assignments, and anti-racist practice examines how routine practices place unequal burdens by race (NASW Code 3.09(e); 1.05(c)). Before redesigning anything, the manager learns from the two social workers how the pattern formed, which assignments came from client requests and what relief they want. It beats B because spreading cases evenly could override clients who asked for a Black social worker, and it would be decided without the people carrying the burden.

B **Incorrect** (trap: Skips assessment). Changing how cases are assigned may be part of the fix. Done now, without knowing which assignments reflect client requests, it could take away a choice some Black clients made and ignore what the two social workers want.

C **Incorrect** (trap: Good but not best). Paying for added duties can be fair and may be part of the solution. On its own, it treats the uneven load as acceptable once paid and skips asking the two social workers what needs to change.

D **Incorrect** (trap: Hands off too soon). Offering support is kind, but sending one worker to the EAP frames a structural assignment problem as personal stress. The pattern stays in place for both workers.

Takeaway: When work assignments burden staff of color unequally, gather the affected staff's input before redesigning, and treat it as an organizational issue, not personal stress.

Sources: NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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14. INT-1539 (rev 2) · Best answer: A

On an inpatient psychiatric unit, a patient with bipolar disorder is set for discharge tomorrow after an overdose attempt. While reviewing aftercare plans, the patient says, 'I still have a month of pills at home. If I lose my job, I'll finish what I started.' The patient's employer has announced layoffs. When the social worker asks directly, the patient says this means taking all the pills to end their life. What should the social worker do **NEXT**?

A. Ask the treatment team to reassess discharge readiness today
B. Add removal of the stored pills to the discharge safety plan
C. Explore the patient's worries about losing the job

Why each option is right or wrong

A **Correct.** Any new suicidal ideation, especially at discharge, calls for reassessment, and discharge rests on the team's judgment that the patient is ready (SAMHSA SAFE-T). The social worker has already asked directly: the patient confirms a plan to overdose on pills at home, tied to a layoff that may come soon, after a recent attempt. Suicide risk is sharply elevated in the weeks after psychiatric discharge (Chung et al., 2017), so the team, including the psychiatrist, must reassess readiness today. It beats B because limiting access to means is essential but belongs in a plan built after the team decides whether discharge is still safe.

B **Incorrect** (trap: Right action, wrong time). Means restriction is a core part of any safety plan and will be needed (Stanley & Brown, 2012). Adding it now treats the discharge as settled, when the confirmed plan and intent call readiness itself into question.

C **Incorrect** (trap: Ignores risk). Working on the job worry can help later, within safety planning. But the patient has already confirmed a plan, the means and a likely trigger, so delaying the team's reassessment risks a discharge that no longer fits the risk. Exploring the stressor fits after the team has reviewed discharge.

Takeaway: A newly confirmed suicide plan with access to means before discharge reopens the discharge decision; inform the team before refining aftercare.

Sources: Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., et al. (2017). Suicide rates after discharge from psychiatric facilities: a systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article; Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256-264, making the environment safe; SAMHSA (2009). SAFE-T: Suicide Assessment Five-step Evaluation and Triage pocket card (SMA09-4432), Step 3: suicide inquiry; reassess as circumstances change and at inpatient discharge
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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15. ETH-1707 (rev 1) · Best answer: A

An autistic adult starts counseling at an agency whose secure online system supports both video and live written messaging. The client explains that speaking aloud under stress makes them lose words and that video eye contact leads to shutdown, but they write clearly and quickly. The supervisor prefers video so the social worker can observe affect. What is the **BEST** way to provide services?

A. Hold sessions through the system's text chat
B. Hold video sessions on the system with the camera off
C. Refer the client to an autism specialty program

Why each option is right or wrong

A **Correct.** Accessible practice adapts the service to how a neurodivergent client communicates best, treating this as a difference rather than a deficit. The client has named the barrier, live speech under stress, and a channel that works, writing, and the agency's secure system supports it. Technology standards call for assessing whether an electronic format suits each client, including people with differing abilities (Technology Standards 2.01, 2.05), and the ADA requires reasonable changes in practices (28 CFR 36.302(a)). It beats B because turning off the camera removes eye contact but still demands live speech.

B **Incorrect** (trap: Misuses the technique). Removing video helps with eye contact and might suit a client whose only difficulty was being seen. It still requires speaking aloud in real time, which the client says causes words to drop out under stress.

C **Incorrect** (trap: Hands off too soon). A specialty program helps when needs exceed the worker's skills. This client needs a change in format the agency already offers, so referring out turns a simple accommodation into a barrier to service.

Takeaway: Match the service format to a neurodivergent client's stated communication strengths when a secure option exists; accommodation is not a lesser service.

Sources: NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.01; Standard 2.05; ADA Title III regulations, 28 CFR 36.302(a)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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16. ASM-1614 (rev 1) · Best answer: B

A client is referred to a mixed-gender trauma recovery group after a sexual assault. She tells the social worker that her faith forbids discussing sexual matters in the presence of men, and she wants trauma treatment. The agency also runs a trauma group for women that has openings. How should the social worker adjust the plan?

A. Encourage her to try the mixed group for a few weeks
B. Switch her referral to the female survivors' group
C. Ask the mixed group to agree to avoid sexual details

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Pressing her to try the mixed group asks her to act against her faith and treats a religious value as an obstacle to overcome. It may lead her to drop out of trauma care altogether.

B **Correct.** Trauma-informed care attends to cultural and gender issues and offers gender-responsive services (SAMHSA, 2014), and social workers provide culturally informed services (NASW Code 1.05(b)). For this client, discussing sexual matters with men present conflicts with her faith, so a mixed group would block the disclosure that trauma work requires. The group for women offers the same treatment in a format that fits her values, and it has openings. It beats C because limiting what members may discuss narrows everyone's recovery work and still leaves the client in a setting her faith forbids.

C **Incorrect** (trap: Misuses the technique). Restricting what other members may discuss changes the group's purpose for everyone and still places the client in a mixed setting. Adjusting her placement respects her value without limiting others' recovery.

Takeaway: When a cultural or religious value conflicts with a service format, adapt the format so effective care stays accessible.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: Cultural, Historical, and Gender Issues; NASW Code of Ethics (2021), 1.05(b)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.4 Culture in intervention planning · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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17. ETH-1069 (rev 1) · Best answer: D

An associate social worker at a community clinic notices that claims for the associate's therapy sessions list the licensed supervisor as the provider who delivered them. The health plan's contract requires claims to name the person who actually provided the service. The associate raised this with the supervisor, who replied, 'Everyone does it.' What should the associate do **NEXT**?

A. Report the billing practice to the health plan's fraud unit
B. Ask the supervisor to co-sign each note to support the claims
C. Keep providing sessions and document the objection in a memo
D. Take the concern to the organization's compliance officer

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reporting to the payer may become necessary if the agency does not correct the practice. Going outside first skips the internal compliance office, which can stop the billing and correct past claims faster.

B **Incorrect** (trap: Misuses the technique). Co-signing shows supervisory review, but it does not make the supervisor the person who delivered the sessions. The claims would still misstate the provider (3.05).

C **Incorrect** (trap: Misreads law or ethics). A private memo records disagreement but leaves the inaccurate billing in place. Continuing in silence amounts to condoning fraud (4.04).

D **Correct.** Billing must accurately show the nature of services and who provided them (NASW Code 3.05), and social workers must not take part in or condone fraud (4.04). They also take reasonable steps to make employer practices consistent with the Code (3.09(d)). The associate already raised it with the supervisor without result, so the next step is the agency's formal compliance channel, which can correct current and past claims. D beats A because an outside report is premature while an internal channel that can fix the problem is untried; it becomes appropriate if the agency fails to act.

Takeaway: When billing misstates who provided services and a supervisor dismisses it, take it to the agency's compliance channel before going outside.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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18. INT-1137 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A client asks an employee assistance social worker for help with anger. For 3 months, a coworker has made slurs about the client's background, and two other staff members have heard them. The client has not acted aggressively but lies awake furious and wants the remarks to stop. How should the social worker **BEST** help with the anger?

A. Teach slow breathing for the moments the coworker speaks
B. Help the client plan an assertive report through work channels
C. Help the client consider other reasons for the coworker's remarks

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Slow breathing is a sound arousal-control skill and could be added. On its own, it treats the client's reaction as the whole problem while repeated, witnessed slurs continue, and the client has said the goal is to make them stop.

B **Correct.** Anger can be a moral response to injustice, and anger management aims to express it assertively rather than aggressively, not to erase it (Reilly & Shopshire, 2019). The slurs are real, repeated, and witnessed, and the client wants them to stop, so the anger points to a problem that action can change. Planning an assertive report, with the witnesses as support, turns the anger into effective action. It beats A because breathing eases arousal in the moment but leaves the harassment, the source of the anger, untouched.

C **Incorrect** (trap: Misuses the technique). Looking for other explanations helps when anger rests on a hostile reading of an ambiguous event. These remarks are slurs that others also heard, so questioning the client's reading would invalidate an accurate perception.

Takeaway: When anger responds to real, ongoing mistreatment, help the client act on it assertively instead of only calming it or questioning the perception.

Sources: Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), anger as a normal emotion; Sessions 7 and 8: assertiveness training and the conflict resolution model

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.20 Helping clients manage anger · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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19. ETH-1507 (rev 1) · Best answer: C

A hospital social worker is helping a 79-year-old patient choose a skilled nursing facility for rehabilitation after a stroke. The social worker's partner is the admissions director at one of the 5 facilities with open beds. The patient asks which facility the social worker would pick. What is the **BEST** response?

A. Recommend the partner's facility, which has strong ratings

B. Leave the partner's facility off the list of options

C. Disclose the connection and review each option neutrally

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Even with strong ratings, a recommendation shaped by the partner's job is the kind of conflict the Code warns about. The patient cannot weigh the advice without knowing about the connection.

B **Incorrect** (trap: Good but not best). Removing the facility avoids the appearance of favoritism, but it hides the conflict and takes away an option that might suit the patient best. Disclosure lets the patient decide with full information.

C **Correct.** Social workers inform clients of real or potential conflicts of interest and resolve them in the client's interest (NASW Code 1.06(a)). The partner's job could color any advice, so the social worker names the connection and then reviews all 5 facilities against the patient's own priorities, such as location, therapy services and ratings. This keeps the patient's choice informed and free. It beats B because quietly dropping a

facility hides the conflict and narrows the options; that facility might be the best fit (Reamer, 2020).

Takeaway: Disclose real or potential conflicts of interest and keep the client's choice informed and free.

Sources: NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, conflicts of interest
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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20. INT-1054 (rev 1) · Best answer: C

A couple asks for help with frequent arguments. In the individual screening session, one partner says the other checks their phone daily, controls all the money, and once blocked the door to keep them from leaving an argument. The client says they fear the partner's reaction to anything said in joint sessions but wants to 'work on the relationship.' What is the **BEST** response?

A. Begin joint sessions with firm ground rules for respectful talk
B. Explore the client's hopes for the relationship in more depth
C. Refer the client to separate services addressing relationship abuse

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Ground rules cannot make joint sessions safe when one partner fears the other's reaction. With signs of coercive control, what the client says in session can lead to retaliation at home. Joint work may be considered only for low-level conflict in which both partners feel safe.

B **Incorrect** (trap: Good but not best). Exploring goals is normally part of screening, but this client has already described control, a blocked exit and fear. Those facts decide the treatment format now; the client's hopes can be explored further within individual services.

C **Correct.** Individual screening exists to find intimate partner violence before conjoint work begins. Phone monitoring, financial control, a blocked exit and fear of the partner's reaction point to coercive control, a pattern in which joint therapy can raise the client's risk after sessions. Individual, violence-informed services with safety planning protect the client, and the worker does not reveal the disclosure to the other partner (SAMHSA TIP 25). It beats B because the client's hopes matter, but the facts already establish the danger, and more exploration only delays protection.

Takeaway: When screening reveals coercive control or fear of a partner, replace conjoint therapy with individual, violence-informed services and safety planning.

Sources: SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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21. ASM-1137 (rev 1) · Best answer: A

On a home visit, an adult child reports that their 74-year-old parent with Parkinson's disease has begun seeing small animals in the corners of rooms. The parent knows the animals are not real and is not frightened. A Parkinson's medication dose was raised 3 weeks ago. The adult child wants the parent admitted to a psychiatric hospital. What is the social worker's **BEST** action?

A. Help the family tell the neurologist about the new symptoms
B. Support a voluntary psychiatric admission for a full evaluation
C. Reassure the family that hallucinations are expected with the disease

Why each option is right or wrong

A **Correct.** Psychotic symptoms in a medical illness are first evaluated by the treating physician for medical and medication causes. Visual hallucinations are common in Parkinson's disease, insight is often kept early, and a recent dose increase is a possible trigger the neurologist should review. DSM-5-TR recognizes both psychosis due to another medical condition and medication-induced psychosis. It beats B because the parent is calm, aware the animals are not real and safe at home; admission is more restrictive and bypasses the physician who manages the medications (DSM-5-TR).

B **Incorrect** (trap: Skips assessment). Psychiatric admission may be needed if hallucinations bring danger, severe distress or loss of insight. A calm parent with preserved insight and a possible medical trigger does not need a more restrictive setting first.

C **Incorrect** (trap: Good but not best). Hallucinations are indeed common in Parkinson's disease, but they still warrant review, since a medication change, infection or delirium may be contributing and treatable. Reassurance alone delays that review.

Takeaway: New psychotic symptoms in a medical illness go first to the treating physician to review medical and medication causes.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Psychotic Disorder Due to Another Medical Condition; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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22. ETH-1135 (rev 2) · Best answer: B

A patient on an inpatient psychiatric unit has schizophrenia and is preparing for discharge. The team suggests that the patient's parent sign consent for a supported housing program. The patient has no guardian. In a capacity evaluation, the psychiatrist documents that the patient can describe the program, weigh it against living alone and state a choice. What should the social worker do **NEXT**?

A. Ask the parent to sign the program consent as the team suggested
B. Review the program with the patient and obtain their own consent
C. Ask the patient and parent to sign the consent together

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A parent who is not a guardian has no legal authority to consent for an adult patient. Third-party consent is for clients who lack capacity, and this patient's capacity has been documented.

B **Correct.** Adults are presumed able to make their own decisions, and a diagnosis alone does not establish incapacity (Appelbaum, 2007). The psychiatrist has documented that this patient understands the program, weighs options and states a choice, so the patient gives informed consent in clear language with a chance to ask questions (NASW Code 1.03(a)). Third-party consent is for clients who lack capacity (1.03(c)). B beats C because a parent's co-signature adds no legal authority and suggests the patient cannot decide alone; the parent can be involved in planning if the patient wishes.

C **Incorrect** (trap: Overrides the client's choice). Joint signing looks like a respectful way to include family, but the parent has no legal role in this adult's consent, and a co-signature implies the patient's own consent is not enough. The parent can join discharge planning if the patient wants that.

Takeaway: A psychiatric diagnosis is not incapacity; when capacity is documented, the client gives their own informed consent.

Sources: NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(c); Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, presumption of competence; diagnosis alone not determinative

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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23. ETH-1027 (rev 3) · Best answer: D

A 28-year-old client with moderate intellectual developmental disorder lives in supported housing. The client's legal guardian, a sibling, has signed consent for the client to attend a sheltered day program five days a week. In planning meetings, which the guardian has not attended, the client says again and again that they want to keep their part-time grocery job instead. What should the social worker do **FIRST**?

A. Start the day program, since the guardian has consented
B. Treat the client's preference as overriding the guardian
C. Petition the court to review the guardianship
D. Bring the client's stated wishes to the guardian

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The guardian does hold legal authority, so enrolling can look correct. Acting on that consent while the client's clear objection has not reached the decision-maker skips the worker's duty to see that the surrogate's choice reflects the client's wishes.

B **Incorrect** (trap: Misreads law or ethics). The client's voice matters, but a guardian holds legal authority for this decision. Treating the client's preference as automatically overriding it misreads the law; the worker's role is to make sure the guardian hears and weighs it.

C **Incorrect** (trap: Skips assessment). Court review is a serious step for a guardian who acts against the person's interests or refuses to consider their wishes. Nothing yet shows the guardian has heard the objection, so a petition comes far too early.

D **Correct.** When a client cannot give informed consent, the social worker seeks permission from an appropriate third party, explains matters at the client's level, and makes sure the third party acts in line with the client's wishes and interests (NASW Code 1.03(c)). The client's repeated, clear preference for the job is exactly what the guardian must weigh, so the worker first brings it forward (NASW Code 1.14). D beats A because the guardian's signature is legally valid, but enrolling before the guardian considers the client's objection skips that duty.

Takeaway: Surrogate consent should reflect the client's wishes and interests; bring the client's stated preferences to the decision-maker before acting.

Sources: NASW Code of Ethics (2021), 1.03(c); NASW Code of Ethics (2021), 1.14
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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24. INT-1554 (rev 1) · Best answer: A

A patient with advanced heart failure is told that no further treatment will extend life. The patient speaks openly about the diagnosis and has updated a will. The patient then starts following a strict low-salt diet for the first time and asks the palliative care social worker whether 'being perfect from now on' could earn enough time to attend a grandchild's graduation in 2 years. Which of Kubler-Ross's stages is **MOST** likely reflected?

A. Bargaining
B. Denial
C. Acceptance
D. Depression

Why each option is right or wrong

A **Correct.** Bargaining is an attempt to postpone death by striking a deal, often with God or fate, in exchange for good behavior (Kubler-Ross, 1969). This patient accepts the diagnosis but hopes perfect diet control will earn time to reach a valued milestone. It beats C because the will and open talk suggest acceptance, yet trying to trade behavior for more time shows the patient has not made peace with the outcome. Kubler-Ross described these reactions as overlapping, not a fixed sequence.

B **Incorrect** (trap: Wrong model or concept). Denial shows as rejecting or ignoring the diagnosis, such as insisting the doctors are wrong. This patient names the illness openly and has updated a will, which shows the reality is recognized.

C **Incorrect** (trap: Wrong model or concept). Updating a will and talking openly fit acceptance, so this option is tempting. Acceptance, however, involves calm readiness rather than an effort to win more time through perfect behavior.

D **Incorrect** (trap: Wrong model or concept). Depression in this model involves sadness, withdrawal and grieving coming losses. This patient is energized and hopeful, focused on a deal for more time rather than on despair.

Takeaway: Bargaining offers good behavior in exchange for more time; it can coexist with open acknowledgment because the stages overlap.

Sources: Kubler-Ross, E. (1969). On Death and Dying. Macmillan, the third stage: bargaining
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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25. INT-1080 (rev 1) · Best answer: B

At a public benefits office, a client whose food assistance was denied is pacing near the front desk, shouting that staff 'never help anyone.' The client's hands are empty, and other people in the lobby have moved away. The social worker approaches. What should the social worker do **FIRST**?

A. Tell the client security will be called unless they sit down
B. Speak calmly from a safe distance and offer a quieter room
C. Ask the client to explain why the application was denied
D. Ask security staff to escort the client out of the building

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Clear limits are part of de-escalation, but leading with a threat is provocative and tends to raise agitation. Limits fit later, if calm contact and choices have not worked or behavior becomes unsafe.

B **Correct.** Verbal de-escalation starts by respecting personal space, establishing calm verbal contact and offering choices, before problem solving or limit setting (Richmond et al., 2012). The client is agitated but holding nothing, so the worker can approach and lower the intensity. A quieter room also removes the audience. It beats C because the denial will need review, but a person this aroused cannot take in details; calming comes before problem solving.

C **Incorrect** (trap: Right action, wrong time). Reviewing the denial is the eventual goal, but problem solving requires a calmer state. Asking for detailed reasons while the client is shouting is likely to feed the anger rather than resolve it.

D **Incorrect** (trap: Skips assessment). Removal is appropriate if the client threatens or harms someone. Here the client is loud and upset but is not threatening anyone, so calling security skips less restrictive steps and may provoke a confrontation.

Takeaway: With an agitated client, start with calm verbal contact, personal space and choices; problem solving and firm limits come after arousal drops.

Sources: Richmond, J. S., et al. (2012). Verbal de-escalation of the agitated patient: Consensus statement of the American Association for Emergency Psychiatry Project BETA De-escalation Workgroup. Western Journal of Emergency Medicine, 13(1), 17-25, domains: respect personal space; establish verbal contact; offer choices
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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26. INT-1528 (rev 3) · Best answer: A

A social worker leads a problem-solving skills group at a clubhouse for adults with serious mental illness. Members are suggesting ways to get to a new job site after a bus route ended. One member responds to each idea as it is offered, saying, 'That won't work.' What should the social worker do **FIRST**?

A. Ask members to list every idea before judging any of them
B. Invite the critical member to offer an idea of their own
C. Ask the member to explain what makes each idea unworkable

Why each option is right or wrong

A **Correct.** When generating alternatives, structured problem solving defers judgment, because criticism cuts the number and variety of options (D'Zurilla & Nezu, 2007). The member's running critique is shutting ideas down, so the worker first restates the ground rule: list everything, then evaluate. Skills training for serious mental illness teaches the same order (Bellack et al., 2004). It beats C because weighing each idea's drawbacks belongs to the next step, decision making, and doing it now ends idea generation early.

B **Incorrect** (trap: Good but not best). Drawing the critic into generating ideas is a sound engagement move and may help. It leaves the group without the ground rule, though, so the next suggestion can still be shot down; restating 'list first, judge later' protects every member's ideas.

C **Incorrect** (trap: Right action, wrong time). Examining drawbacks is part of the decision-making step that follows. Inviting it now rewards the critique and stops idea generation. The member's concerns become useful once the list is complete.

Takeaway: When generating solutions, list all ideas before evaluating any; early criticism narrows the options.

Sources: D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills; Bellack, Mueser, Gingerich & Agresta (2004). Social Skills Training for Schizophrenia (2nd ed.). Guilford Press, problem-solving skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.16 Structured problem-solving approaches · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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27. INT-1084 (rev 2) · Best answer: D

In a hospital, a patient who disclosed a past sexual assault becomes panicky before a needed physical exam, gripping the bed rail and saying, 'I can't do this if I don't know what's coming.' The physician asks the social worker for help. What is the **MOST** appropriate action?

A. Suggest that the physician order a sedative first
B. Ask the physician to postpone the exam until the patient feels ready
C. Reassure the patient that the staff are skilled professionals
D. Arrange for each step to be explained and a way to pause

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Medication decisions belong to the physician and patient, and a sedative skips the patient's own stated need for information. Trauma-informed steps that restore predictability and control should come first.

B **Incorrect** (trap: Good but not best). Postponing respects the distress, but the exam is needed and the patient has named what would make it possible: knowing what is coming. Delay leaves the fear in place and may make the next attempt harder. Postponing fits if the patient still declines once each step is explained.

C **Incorrect** (trap: Good but not best). Reassurance is well meant, but the panic comes from not knowing what will happen and feeling out of control. Telling the patient the staff are skilled does not supply the predictability the patient asked for.

D **Correct.** Trauma-informed care builds safety, transparency and a sense of control, and avoids repeating experiences of powerlessness (SAMHSA, 2014). The patient states exactly what is needed: knowing what is coming. Explaining each step before it happens and agreeing on a signal to pause gives predictability and voice. It beats C because reassurance about staff skill is kind but does not change the loss of control that drives the panic; the patient asked for information, not credentials.

Takeaway: Trauma-informed care restores predictability and control: explain each step in advance and give the person a way to pause.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: safety; trustworthiness and transparency; empowerment, voice and choice
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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28. ETH-1082 (rev 1) · Best answer: C

A foster parent tells a child welfare social worker that a 14-year-old in care has been meeting a birth parent at a mall on weekends. The court allows only supervised visits. The foster parent adds that the youth asked them not to tell anyone. The social worker has a scheduled meeting with the youth this afternoon. What should the social worker do **FIRST**?

A. Promise the youth secrecy if the meetings stop
B. Notify the court that the visit order was violated
C. Ask the youth about the meetings and how safe they felt
D. Report the foster parent's account to the supervisor

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A promise of secrecy cannot be kept, because the worker has duties to the agency and the court. Clients should hear the real limits of confidentiality, not a conditional promise (NASW Code 1.07(e)).

B **Incorrect** (trap: Skips assessment). The court may need to know, but filing a notice on a secondhand report skips fact-finding and the agency's own process. It is premature at this point.

C **Correct.** Working through a dilemma starts with clarifying the facts and the issues at stake (Reamer, 2018). This report is secondhand, and the court's supervision order

signals a possible safety concern, so the youth's own account and sense of safety come first, and the meeting is only hours away. C beats D because the supervisor needs to know, but a same-day conversation gives the supervisor verified facts and safety information to act on instead of an untested account.

D **Incorrect** (trap: Right action, wrong time). Supervisor involvement is expected for a possible court-order violation, and it should follow quickly. Passing on an unverified account before speaking with the youth, who will be seen today, gives the supervisor less to act on.

Takeaway: When a dilemma arises from secondhand information, clarify the facts and any safety concern before choosing or reporting a course of action.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework; NASW Code of Ethics (2021), 1.07(e)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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29. INT-1014 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A client staying in an emergency shelter meets with a social worker who uses solution-focused brief therapy. Today the client says the week was awful, nothing improved, and the job search went nowhere. Which question is **MOST** appropriate for the social worker to ask?

A. 'How have you kept things from getting even worse?'
B. 'If a miracle happened tonight, what would be different?'
C. 'What do you think is keeping you stuck right now?'

Why each option is right or wrong

A **Correct.** Solution-focused brief therapy uses coping questions when a client reports that nothing has improved. Asking how the client has kept things from getting worse draws out the strengths and effort the client is already using, even in a hard week (de Shazer et al., 2007). It beats B because the miracle question invites the client to picture a preferred future, which can feel out of reach to someone describing a week of setbacks; the coping question meets the client where they are.

B **Incorrect** (trap: Misuses the technique). The miracle question is a core SFBT technique for describing goals. Asked right after the client reports a terrible week, it can sound dismissive. It fits better once the client feels heard and can picture change.

C **Incorrect** (trap: Wrong model or concept). Asking what keeps the client stuck invites analysis of the problem's causes. SFBT deliberately avoids problem analysis and builds on what the client is already doing that works.

Takeaway: In SFBT, when a client reports no progress, coping questions uncover the strengths that kept things from getting worse.

Sources: de Shazer, S., Dolan, Y., Korman, H., Trepper, T., McCollum, E., & Berg, I. K. (2007). More Than Miracles: The State of the Art of Solution-Focused Brief Therapy. Haworth Press, coping questions
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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30. ETH-1651 (rev 2) · Best answer: A

A client with schizophrenia has had a representative payee for Social Security benefits since a hospital stay 5 years ago. For the past 18 months, the client has worked part-time, paid rent from wages on time and kept a written budget. The client tells the social worker, 'I want my check sent to me again.' What is the **MOST** appropriate response?

A. Help the client gather evidence and request direct payment
B. Ask the payee to give the client a larger weekly allowance
C. Advise the client to keep the payee until the next yearly review
D. Arrange a formal assessment of the client's money skills first

Why each option is right or wrong

A **Correct.** Self-determination includes managing one's own money when able (NASW Code 1.02). The Social Security Administration says a beneficiary with a payee can receive benefits directly by showing the ability to manage them, for example with a doctor's statement or other evidence (SSA payee FAQs). Eighteen months of on-time rent and a written budget are that kind of evidence. It beats B because a larger allowance keeps the payee in control and treats a rights request as a spending problem. The client should also know that Social Security may review disability status when a condition improves.

B **Incorrect** (trap: Good but not best). A bigger allowance may ease day-to-day frustration and is a reasonable interim step if the client wants it. It leaves the payee in charge, though, and sidesteps what the client asked for: control of their own benefits.

C **Incorrect** (trap: Misreads law or ethics). No rule makes a beneficiary wait for a review. Social Security's payee guidance sets no waiting period: the beneficiary can ask to manage their own benefits with evidence such as a doctor's statement, and a payee decision can be appealed.

D **Incorrect** (trap: Good but not best). A formal skills assessment would make sense with no track record to go on. Here 18 months of budgeting and on-time rent already show the ability, so another assessment adds a hurdle the client does not need to clear.

Takeaway: When a client with a payee shows they can manage money, help them pursue direct payment rather than keep the restriction in place.

Sources: NASW Code of Ethics (2021), 1.02; Social Security Administration. FAQs for Beneficiaries Who Have a Representative Payee (ssa.gov/payee), accessed 2026., managing your own benefits; appeals
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

31. ASM-1156 (rev 2) · Best answer: C

Teachers ask a high school social worker about a 16-year-old with strong past grades who, over the last 3 weeks, has become restless, irritable, anxious, and unable to focus in morning classes. The changes began when the school installed vape sensors in the restrooms. A classmate says the student used to vape between every class. The student now asks to leave campus at lunch and comes back calmer. Which explanation is **MOST** likely?

A. Attention-deficit/hyperactivity disorder
B. Generalized anxiety disorder
C. Tobacco withdrawal

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). ADHD is a common referral question when focus drops, but its symptoms must begin in childhood and show up across settings. A sudden change at 16 in a student with a strong record, tied to a specific school event, does not fit.

B **Incorrect** (trap: Near-miss diagnosis). Generalized anxiety brings restlessness and poor concentration, but it centers on months of hard-to-control worry about many things. These symptoms began abruptly with the sensors and ease after time off campus, a pattern that points to a substance.

C **Correct.** Tobacco withdrawal follows stopping or sharply cutting back daily nicotine use and brings irritability, restlessness, anxiety and trouble concentrating, often within a day (DSM-5-TR). A student who vaped between every class likely had daily use, and the restroom sensors cut it off during the school day, and the symptoms ease after leaving campus at lunch. It beats A because ADHD symptoms must start in childhood and appear across settings, while these began 3 weeks ago in a student with a strong record.

Takeaway: New irritability, restlessness and poor focus that start when daily nicotine use is interrupted suggest tobacco withdrawal, not a new disorder.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Tobacco Withdrawal; DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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32. ETH-1139 (rev 2) · Best answer: A

A new client has filed a lawsuit against a former employer, claiming emotional distress from workplace harassment. The client wants therapy focused on the harassment and its effects on sleep and mood. At intake, the client asks what the social worker will write down. What is the **BEST** response?

A. Explain that the lawsuit may lead to requests for the records
B. Recommend waiting to start therapy until the lawsuit is resolved
C. Agree to keep notes brief and leave out harassment details
D. Assure the client that therapy records are privileged in court

Why each option is right or wrong

A **Correct.** Informed consent includes the risks of services in clear language, and the limits of confidentiality are discussed early (NASW Code 1.03(a), 1.07(e)). When a client claims emotional distress in a lawsuit, the other side may seek therapy records, and privilege may not protect them. Knowing this, the client can decide what to discuss and can consult their attorney. A beats D because promising privilege overstates a protection that a client can lose by putting their mental state at issue.

B **Incorrect** (trap: Too directive). Delaying care would leave the client without help for sleep and mood problems now. Whether to start despite the legal risk is the client's decision once informed; the worker's role is to inform, not to steer.

C **Incorrect** (trap: Misreads law or ethics). Brief notes can protect privacy, but records must be accurate and reflect the services provided (3.04(a)). Leaving out the main focus of treatment to shape a lawsuit makes the record misleading.

D **Incorrect** (trap: Misreads law or ethics). Privilege belongs to the client, but a client who claims emotional distress in court may waive it for related records. Promising protection gives the client false security about a real risk.

Takeaway: Before treatment, tell clients in litigation that their records may be sought, so they can decide with that knowledge.

Sources: NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.07(e); NASW Code of Ethics (2021), 3.04(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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33. ASM-1612 (rev 1) · Best answer: B

A juvenile court asks a social worker to start a weekly group for eight 14- and 15-year-olds on probation for fighting and theft. The court says a group will serve more youth at lower cost. Most of the youth already spend time together, and several have parents who are willing to take part in services. What is the **BEST** recommendation?

A. Run the group with a structured anger-management curriculum
B. Offer family-based work instead of a peer group
C. Hold individual sessions focused on insight into behavior

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Structure and skill content help, but the format is the problem: placing teens on probation for fighting and theft together can teach and reward problem behavior. A peer group is a better fit for lower-risk youth.

B **Correct.** Development shapes modality. Adolescents are highly responsive to peers, and grouping youth with conduct problems can increase problem behavior through peer reinforcement, often called deviancy training (Dishion, McCord & Poulin, 1999). These youth already spend time together, which raises that risk, and willing parents make family-based work feasible. It beats A because even a structured curriculum still places high-risk teens together, and the cost savings do not offset the risk of worsening behavior.

C **Incorrect** (trap: Good but not best). Individual sessions avoid peer aggregation, but insight-focused talk is a weak fit for adolescent conduct problems and leaves out the

parents who are ready to help. Family involvement offers more leverage over daily behavior.

Takeaway: Avoid grouping high-risk adolescents with conduct problems; peer aggregation can worsen behavior, so favor family-based approaches when parents can engage.

Sources: Dishion, T. J., McCord, J., & Poulin, F. (1999). When interventions harm: Peer groups and problem behavior. American Psychologist, 54(9), 755-764, whole article
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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34. INT-1121 (rev 2) · Best answer: B

During a home visit, an adult child caring for a parent with major neurocognitive disorder says the parent asks the same question 20 times an hour. The social worker acknowledges how draining that sounds. The caregiver replies, 'I think it's on purpose, to get under my skin,' and says they have started snapping at the parent. What is the **MOST** appropriate response?

A. Suggest gently reminding the parent that the question was answered
B. Explain that memory loss keeps the parent from recalling asking
C. Reflect the caregiver's frustration and exhaustion

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Pointing out that the question was already answered seems logical, but the parent cannot remember asking, so the reminder usually adds confusion and distress. Caregiver guidance advises against arguing or using logic.

B **Correct.** Psychoeducation corrects a misunderstanding that is driving harm. The worker has already acknowledged the strain, and the caregiver still believes the repetition is deliberate; that belief is fueling the snapping. Explaining that the disease damages memory, so the parent does not recall having asked, reframes the behavior as a symptom and leads to strategies such as answering calmly, responding to the worry behind the question, or posting a written answer (Alzheimer's Association). It beats C because the strain has already been acknowledged; reflecting it again leaves the 'on purpose' belief in place and keeps the conflict going.

C **Incorrect** (trap: Good but not best). Caregiver frustration is real and deserves empathy, but the social worker has just acknowledged it. Reflecting it again leaves the belief that the parent is doing this on purpose untouched, so the snapping is likely to continue. Reflection fits as the opening response before the misattribution is voiced.

Takeaway: When a caregiver misreads a dementia symptom as deliberate, explain the symptom and offer coping strategies; empathy alone leaves the misattribution.

Sources: Alzheimer's Association. Repetition (Caregiving: Stages and Behaviors), causes of repetition and caregiver tips
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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35. ASM-1117 (rev 1) · Best answer: B

A client with schizophrenia served by an assertive community treatment team asks to reduce home visits from daily to weekly. For 8 months, the client has kept a daily symptom log, twice called the team when sleep loss and suspicious thoughts began, and followed the agreed steps each time. A teammate wants to keep daily visits because of a hospitalization 2 years ago. What is the **BEST** response?

A. Keep daily visits until the client has a full year of stability
B. Plan a gradual step-down built on the client's warning-sign log
C. Transfer the client from the team to routine clinic care

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Caution after a past hospitalization is understandable. Setting an arbitrary waiting period, though, disregards 8 months of shown self-monitoring and overrides a reasonable request without a current reason.

B **Correct.** Self-determination includes the right to less intensive services when the client can manage safely (NASW Code 1.02). Monitoring one's own early warning signs is a recognized relapse-prevention skill (Birchwood et al., 1989), and this client has shown it for 8 months, twice catching early signs and acting on the plan. A gradual step-down that keeps the log and call plan in place honors the request while managing risk. It beats A because holding to daily visits rests on a 2-year-old event and ignores current evidence that the client monitors symptoms reliably.

C **Incorrect** (trap: Hands off too soon). Moving straight to routine clinic care removes intensive support all at once. A stepped reduction lets the team confirm that self-monitoring holds up with less contact before any transfer.

Takeaway: Shown ability to monitor one's own warning signs supports honoring a request for less intensive care through a planned step-down.

Sources: NASW Code of Ethics (2021), 1.02; Birchwood, M., Smith, J., Macmillan, F., Hogg, B., Prasad, R., Harvey, C., & Bering, S. (1989). Predicting relapse in schizophrenia: The development and implementation of an early signs monitoring system using patients and families as observers. Psychological Medicine, 19(3), 649-656, Early Signs Scale

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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36. ASM-1668 (rev 1) · Best answer: C

In a second video session, a client with moderate depression agrees that setting goals would help but says, 'I honestly don't know what I want. I just want to feel less stuck.' The client is engaged and willing to keep talking about it. What is the **BEST** way for the social worker to proceed?

A. Offer a list of common goals for people with depression
B. Wait to set goals until the client's mood improves
C. Ask how the client's days would differ if the problem lifted
D. Set a goal of lowering the client's PHQ-9 score

Why each option is right or wrong

A **Incorrect** (trap: Too directive). A menu of goals can help as a last resort, but offering it first steers the client toward the worker's ideas. Goals the client shapes are more meaningful and more likely to be pursued.

B **Incorrect** (trap: Good but not best). Waiting for mood to lift delays help, and goal setting itself can build hope. Depression is a reason to make goals small and concrete, not a reason to put planning off.

C **Correct.** Solution-focused questions such as the miracle question help clients who cannot name goals picture concrete signs that life is better, and those signs can become goals in the client's own words (de Shazer, 1988). This client wants to feel 'less stuck' but cannot yet say what that looks like. Asking how daily life would be different if the problem lifted turns a vague wish into observable changes. It beats A because a list of common goals puts the worker's ideas in place of the client's and can yield goals the client accepts but does not own.

D **Incorrect** (trap: Misuses the technique). Tracking a depression measure is a useful outcome indicator, but choosing it for the client skips the client's own picture of change. It can sit alongside goals the client defines.

Takeaway: When clients cannot name goals, ask solution-focused questions that help them describe life with the problem resolved.

Sources: de Shazer, S. (1988). Clues: Investigating Solutions in Brief Therapy. W. W. Norton, miracle question **Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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37. ETH-1028 (rev 1) · Best answer: C

Before a first session at an outpatient clinic, a social worker's colleague suggests looking up the new client's public social media accounts 'to get the full picture.' The referral describes mild anxiety about a job change. How should the social worker handle the suggestion?

A. Look at public posts, which anyone can already see

B. Search now, then tell the client what turned up

C. Seek the client's consent before doing any search

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Public availability does not cancel the consent rule. The Code treats an electronic search about a client as needing consent unless safety or another compelling reason applies, and mild job-related anxiety offers neither.

B **Incorrect** (trap: Misreads law or ethics). Telling the client afterward is honest, but it turns consent into notice. The client loses the chance to decide before the search happens, which is the point of the consent standard.

C **Correct.** Social workers obtain client consent before searching electronically for information about a client, unless the search protects someone from serious, foreseeable and imminent harm or another compelling reason exists (NASW Code 1.03(i)). A referral for mild anxiety about a job change gives no such reason. C beats A

because public visibility does not remove the requirement; the Code asks workers to avoid gathering client information online without a compelling reason and, when appropriate, consent (NASW Code 1.07(q)).

Takeaway: Get a client's consent before searching online for information about them, unless imminent harm or another compelling reason exists.

Sources: NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.07(q)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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38. ASM-1504 (rev 1) · Best answer: A

A parent at a pediatric clinic tells the integrated behavioral health social worker that their 2-year-old has turned 'defiant.' The toddler insists on putting on shoes alone, says 'no' to many requests and has brief tantrums when stopped. The toddler sleeps well, speaks in short phrases and plays happily at child care. What is the **MOST** appropriate response?

A. Explain that these behaviors fit typical toddler development
B. Refer the toddler for a disruptive behavior disorder evaluation
C. Ask the parent to track every tantrum for 2 weeks

Why each option is right or wrong

A **Correct.** Erikson's stage of autonomy versus shame and doubt (about ages 1.5 to 3) centers on the toddler's push to act independently (Erikson, 1963). Insisting on doing things alone, frequent refusals and brief tantrums when stopped are expected at age 2, and the toddler's good sleep, language and play support typical development. It beats C because a tantrum log adds little when the stem already shows a normal pattern; it would help if tantrums were long, injurious or worsening. Guidance such as offering limited choices can follow.

B **Incorrect** (trap: Hands off too soon). A disruptive behavior evaluation suits persistent, severe or impairing behavior well beyond age norms. Referring a 2-year-old for normal independence-seeking pathologizes typical development and may alarm the parent without cause.

C **Incorrect** (trap: Good but not best). Tracking tantrums helps when their frequency, length or severity is unclear or concerning. Here the description already fits ordinary toddler behavior, so a 2-week log delays the reassurance and guidance the parent needs.

Takeaway: Toddler insistence on independence, frequent refusals and brief tantrums are typical autonomy-stage behavior; normalize and guide before considering referral.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: autonomy vs. shame and doubt
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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39. ASM-0010 (rev 1) · Best answer: B

A client treated for panic disorder has had no panic attacks for 2 months and uses breathing and exposure skills without prompting. At the planned next-to-last session, the client says they feel nervous about ending and asks to keep meeting weekly 'just in case.' What is the social worker's **BEST** response?

A. Continue weekly sessions until the client feels ready to stop
B. Explore the client's worries and review the gains and relapse plan
C. Resume exposure work because the anxiety signals a relapse

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Open-ended weekly sessions after goals are met can foster dependence and confirm the client's fear of coping alone. A planned step-down or booster session could be discussed instead of continuing by default.

B **Correct.** Lasting symptom relief and independent use of skills are signs of readiness for termination. Anxiety about ending is a common, expected reaction rather than a sign of relapse (Hepworth et al.). The best response explores the worry, reviews what the client has achieved, rehearses a relapse-prevention plan and makes clear how to return if needed. It beats A because extending weekly sessions without a clinical need can build dependence and implies the client cannot cope without the social worker. Services end when they are no longer needed (NASW Code 1.17(a)).

C **Incorrect** (trap: Skips assessment). Nervousness about ending is different from a return of panic attacks. Treating it as relapse misreads a normal termination reaction and undercuts the client's confidence in skills already mastered.

Takeaway: Goals met and skills used independently signal readiness to end; anxiety about ending is normal and is processed, not treated as relapse.

Sources: NASW Code of Ethics (2021), 1.17(a); Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: reactions to ending
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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40. ASM-1159 (rev 2) · Best answer: C

During a mental status exam, the social worker asks a client to spell the word 'world' backward. Which area is the social worker assessing?

A. Remote memory
B. Abstract reasoning
C. Attention and concentration

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Remote memory covers events from years past and is tested by asking about personal history. Spelling backward works on information held in mind right now, so it measures attention rather than stored memory.

B **Incorrect** (trap: Wrong model or concept). Abstract reasoning is tested with similarities or proverb meanings, which ask the client to move from specific facts to general ideas. Spelling backward involves no such step.

C **Correct.** Spelling a short word backward requires holding letters in mind and working through them in reverse, so it tests attention and concentration, as does subtracting 7s from 100 (Kaplan & Sadock). Results help show whether attention is impaired, as in delirium. It beats A because memory tasks ask the client to recall past events or a word list after a delay, not to work with information on the spot.

Takeaway: Serial 7s and spelling 'world' backward test attention and concentration on the mental status exam.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: sensorium and cognition

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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41. ASM-1041 (rev 1) · Best answer: B

A high school social worker meets a 15-year-old cross-country runner referred by a coach. The student has lost a lot of weight over 3 months, skips lunch and runs extra miles. Today the student fainted at practice, and the athletic trainer recorded a very slow resting heart rate. The student says they feel fine. What should the social worker do **FIRST**?

A. Explore the student's thoughts about weight and body shape

B. Have the parents take the student to a doctor right away

C. Refer the student to an eating disorder therapy program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Body image questions are central to assessing a possible eating disorder and will come soon. They follow medical safety when fainting and a slow heart rate point to possible instability.

B **Correct.** Fainting and a very slow resting heart rate in a teen with rapid weight loss and food restriction are signs of medical instability, which can require hospital care in restrictive eating disorders (Hornberger & Lane, 2021). Safety comes first, so the student needs a same-day medical evaluation with parents involved. It beats A because exploring body image is a core part of the eating disorder assessment, but it can wait until the student is medically safe.

C **Incorrect** (trap: Ignores risk). Eating disorder treatment is likely needed, but an outpatient therapy referral leaves the immediate medical risk unaddressed. Medical stability is checked first, and it also guides the level of care.

Takeaway: Fainting or a very slow heart rate in a restricting teen signals medical instability; get same-day medical care before psychological assessment or therapy.

Sources: Hornberger, L. L., Lane, M. A., & AAP Committee on Adolescence (2021). Identification and management of eating disorders in children and adolescents. Pediatrics, 147(1), e2020040279, medical evaluation and indications for hospitalization

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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42. ASM-1166 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

In a nursing facility, a resident with major neurocognitive disorder due to Alzheimer's disease has become agitated in the evenings. The physician proposes starting risperidone. The resident's adult child, who is the health care proxy, has read that such drugs can be risky in dementia and asks the social worker what to do. What is the **MOST** helpful response?

A. Advise the proxy to refuse the drug because of its known risks
B. Help the proxy ask the physician about risks and alternatives
C. Reassure the proxy that the physician has weighed the risks

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). The proxy's concern is valid, but advising refusal is a medical judgment outside the social work role. The decision belongs to the proxy after a discussion with the physician.

B **Correct.** Antipsychotics carry a boxed warning for increased death rates in older adults with dementia-related psychosis. The APA practice guideline advises reviewing nondrug approaches and discussing risks and benefits with the patient's surrogate before starting one (Reus et al., 2016). The social worker can confirm that the concern is a real one and help the proxy prepare questions, supporting an informed decision without giving medical advice. It beats C because reassurance closes off a discussion the proxy is entitled to have.

C **Incorrect** (trap: Good but not best). The physician may well have weighed the risks, but the proxy has the right to understand them before consenting. Reassurance discourages the questions informed consent depends on.

Takeaway: Help surrogates weigh antipsychotic risks in dementia by preparing questions for the prescriber; don't advise for or against the drug.

Sources: Reus, V. I., Fochtmann, L. J., Eyler, A. E., et al. (2016). The American Psychiatric Association practice guideline on the use of antipsychotics to treat agitation or psychosis in patients with dementia. American Journal of Psychiatry, 173(5), 543-546, Guideline statements

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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43. ETH-1511 (rev 3) · Best answer: A

A 14-year-old in residential treatment, who has a history of sexual abuse, greets the social worker with long, full-body hugs before each session. The client says the hugs make them feel safe. Other staff have started to comment on the greetings. What is the social worker's **BEST** response?

A. Offer a side hug or handshake and discuss the change
B. Keep the hugs but make each one brief
C. Step back from hugs and avoid raising the topic

Why each option is right or wrong

A **Correct.** Social workers avoid physical contact that could cause psychological harm and set clear, culturally sensitive boundaries when some contact is appropriate (NASW Code 1.10). For a teen with a sexual-abuse history, prolonged full-body hugs can confuse boundaries, yet a sudden rejection could feel like punishment. Offering a safer greeting and talking it through preserves the client's sense of safety while modeling healthy limits (SAMHSA, 2014). It beats C because pulling away without explanation leaves the client guessing and may feel like rejection by a trusted adult.

B **Incorrect** (trap: Boundary slip or collusion). Shortening the hugs moves toward a limit, which makes this tempting. Full-body hugs with a teen who has been sexually abused still blur what safe touch looks like, and an unexplained change teaches nothing; the social worker, not the client, is responsible for physical boundaries.

C **Incorrect** (trap: Misuses the technique). Ending the hugs is the right direction, but doing it silently is the wrong method. The client may feel rejected or blamed, and the chance to teach and practice a safe greeting is lost.

Takeaway: The social worker sets physical-contact boundaries, offering safer alternatives and discussing them, especially with trauma survivors.

Sources: NASW Code of Ethics (2021), 1.10; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: safety
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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44. ETH-1518 (rev 2) · Best answer: D

Under the NASW Code of Ethics, which condition must be present before a social worker accepts goods or services from a client instead of a fee?

A. The goods match the fee's market value
B. The client's insurance coverage has lapsed
C. A supervisor approves the plan in writing
D. The client proposes it with informed consent

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Equal value can seem fair, but market value is not the Code's test. A barter of matched value can still be coercive or harmful, and the Code's conditions focus on initiative, consent, local acceptance and necessity.

B **Incorrect** (trap: Misreads law or ethics). Lost coverage often prompts barter, but financial hardship is not one of the Code's conditions. The worker should first consider other options, such as a sliding fee scale or referral to lower-cost care.

C **Incorrect** (trap: Misreads law or ethics). Supervisory approval may be wise, and some agencies require it, but it is not a Code condition. The Code places the burden on the social worker and requires the client's own initiative and informed consent.

D **Correct.** The Code says social workers should avoid accepting goods or services as payment. Barter is acceptable only in very limited circumstances: it is accepted among local professionals, essential to providing services, negotiated without coercion, and entered into at the client's initiative with informed consent (NASW Code 1.13(b)). The social worker bears the full burden of showing it is not harmful. It beats C because

supervisory approval may be good agency practice but is not a Code condition and cannot substitute for the client's initiative and consent.

Takeaway: Barter only in very limited cases: client-initiated with informed consent, locally accepted, essential and uncoerced; the worker bears the burden.

Sources: NASW Code of Ethics (2021), 1.13(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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45. ETH-1079 (rev 2) · Best answer: C

A social worker at a family shelter is helping a university team design a study of residents' health needs. To boost enrollment, a team member proposes giving each participant priority on the shelter's waitlist for permanent housing. What is the **BEST** way for the social worker to respond to the proposal?

A. Offer participants added case management hours instead
B. Recommend offering no compensation for taking part
C. Suggest a modest cash payment separate from services

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Extra case management sounds modest and helpful, so this is tempting. It still ties a needed shelter service to joining the study, so residents who depend on the shelter may feel unable to refuse; 5.02(e) calls for consent free of undue inducement or penalty.

B **Incorrect** (trap: Good but not best). Paying participants for their time is not unethical in itself. Dropping all compensation solves the inducement problem but goes further than needed and may undervalue residents' time.

C **Correct.** Research consent must be voluntary and free of coercion or undue influence (45 CFR 46.116(a)(2); NASW Code 5.02(e)). Tying a scarce, life-changing service like housing priority to enrollment would pressure residents to join and make shelter services depend on research. Modest payment for time is common and acceptable when it is not large enough to override judgment. C beats B because removing all compensation goes further than needed; fair payment for time respects participants without coercing them.

Takeaway: Research incentives must never involve access to services or be large enough to override a person's judgment about joining.

Sources: 45 CFR Part 46 (Common Rule), 45 CFR 46.116(a)(2); NASW Code of Ethics (2021), 5.02(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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46. ETH-1182 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A prospective client calls a group practice asking to start weekly individual therapy with the social worker. During the call, the client mentions seeing another mental health provider weekly for the same concerns and adds, 'I haven't told them I'm looking around.' What should the social worker do **FIRST**?

A. Schedule an intake appointment for this week as the client requested
B. Decline until the client ends the current therapy
C. Discuss how adding a provider could affect current care
D. Ask permission to contact the current provider

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Starting services without discussing the existing therapy risks parallel treatment with conflicting approaches. The Code asks the social worker to consider the client's needs carefully before agreeing to provide services.

B **Incorrect** (trap: Overrides the client's choice). Requiring the client to end current therapy first imposes a decision that belongs to the client and may cut off support. The client may have good reasons for seeking a change, which the social worker has not yet heard.

C **Correct.** When someone receiving services from a colleague seeks help, the Code asks the social worker to consider the client's needs carefully and to discuss the current relationship and the benefits and risks of starting with a new provider (3.06(a)). Two concurrent therapies for the same concerns can work at cross-purposes, so this conversation comes before any other step. It beats D because contacting the current provider is a possible later step that depends on what the client wants after this discussion, and on the client's consent.

D **Incorrect** (trap: Right action, wrong time). Coordinating with the current provider may become appropriate, with consent, but it is premature before the social worker understands why the client is seeking a second provider and what the client wants.

Takeaway: When a prospective client is already in treatment elsewhere, first discuss that relationship and the risks and benefits of adding a provider.

Sources: NASW Code of Ethics (2021), 3.06(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

47. INT-1668 (rev 1) · Best answer: C

A community mental health agency wants to learn how satisfied its 900 active clients are. Last year it surveyed clients who attended the annual picnic, and about 150 responded. A board member suggests repeating the picnic survey with more volunteers to collect more forms. Which change would **MOST** improve how well the results represent all active clients?

A. Make the picnic survey anonymous so answers are more candid
B. Double the number of picnic surveys collected
C. Survey a random sample drawn from the full client list

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Anonymity helps clients answer honestly and is worth keeping. It addresses response bias, not who is asked. A survey limited to picnic attendees still misses clients who do not attend, so the results would still not represent the agency.

B **Incorrect** (trap: Misuses the technique). More responses shrink random error but do not fix a biased sampling frame. Doubling the picnic sample gives a more precise estimate of picnic attendees' views, not of all active clients.

C **Correct.** A sample represents a population when every member has a known chance of selection; probability sampling from the full client list removes the bias of surveying only those who attend an event (Rubin & Babbie, 2017). Picnic attendees likely differ from other clients in engagement, mobility, or satisfaction itself. A random sample of all 900, with follow-up to raise response, gives results that generalize. It beats A because anonymity reduces pressure to answer favorably, a different bias, but still leaves out every client who skips the picnic.

Takeaway: Representativeness depends on how a sample is drawn, not its size; draw a probability sample from the whole population.

Sources: Rubin, A., & Babbie, E. R. (2017). Research Methods for Social Work (9th ed.). Cengage, sampling: probability sampling and sampling bias
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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48. ETH-1636 (rev 2) · Best answer: A

At an urban Indian health center, a family tells the social worker that their 19-year-old relative has not been seen or heard from in 2 days, which is out of character. When they called police, an officer said young adults often leave on their own and told them to call back in 3 days. The family asks for help. What should the social worker do **FIRST**?

A. Help the family file a missing person report today
B. Help the family post a missing notice on social media
C. Advise the family to wait 3 days while calling friends
D. Help the family call area hospitals and jails

Why each option is right or wrong

A **Correct.** Federal law requires states to ensure that police impose no waiting period on reports of missing persons under 21 and that reports enter national databases promptly (34 U.S.C. 41307, 41308). Indigenous people who go missing are often dismissed as runaways or left out of police records, which delays searches (Urban Indian Health Institute, 2018). The 19-year-old's silence is out of character, so the report should be taken now. It beats D because calling hospitals and jails is a sensible step that can run alongside, but only an official report starts the database entry and the police search.

B **Incorrect** (trap: Right action, wrong time). Community alerts are a common and useful tool, and Native families often lead their own searches. They should follow, not replace, an official report, which starts database entries and police duties.

C **Incorrect** (trap: Ignores risk). No waiting period applies to a missing person this age, and delay loses time when early action matters most. Accepting the officer's advice repeats a pattern that leaves Indigenous missing persons cases unrecorded.

D **Incorrect** (trap: Right action, wrong time). Checking hospitals and jails is a common early step, and it may quickly locate the young person. Doing it before the report, though, lets another day pass without the database entry and police duties that federal law says should start without a waiting period.

Takeaway: When a young person goes missing, press for an immediate police report; no waiting period applies, and delays fall hardest on Indigenous families.

Sources: National Child Search Assistance Act of 1990 (as amended), 34 U.S.C. 41307, 41308; Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: underreporting in law enforcement records

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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49. INT-1693 (rev 2) · Best answer: A

A family support program gives grocery gift cards to clients in need. One case manager orders the cards, keeps them in a desk drawer, hands them out and records each distribution in a spreadsheet. A routine review finds the records complete. The program director wants to reduce the risk of error or misuse. Which control is **MOST** effective?

A. Give purchasing, custody and recordkeeping to different staff

B. Have a supervisor count the cards against the log each month

C. Refer the arrangement to the compliance officer for investigation

Why each option is right or wrong

A **Correct.** Segregation of duties is a basic internal control: authorizing, handling and recording assets are divided so no one person controls a whole transaction (GAO, 2014). One case manager now orders, stores, distributes and logs the cards, so an error or misuse could go undetected. Dividing these tasks creates built-in checks while keeping cards available to clients. It beats B because a count against the case manager's own log cannot catch cards that were ordered but never recorded; separating ordering, custody and records closes that gap.

B **Incorrect** (trap: Good but not best). An independent count is a real control and better than none. The same person still orders the cards and keeps the log the count is checked against, though, so cards ordered and never logged would not show up, and any problem surfaces only weeks later.

C **Incorrect** (trap: Skips assessment). Investigation is warranted when there is evidence of loss or misuse. The review found complete records, and the director's aim is prevention, so the fix is a structural control rather than an inquiry into one employee.

Takeaway: Prevent fiscal error and misuse by dividing authorization, custody and recording of assets among different people.

Sources: GAO, U.S. Government Accountability Office (2014). Standards for Internal Control in the Federal Government (GAO-14-704G), Principle 10: Design Control Activities, segregation of duties

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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50. INT-1165 (rev 1) · Best answer: B

In the first session of a closed outpatient group for adults coping with anxiety, introductions begin. One member starts describing childhood sexual abuse in vivid detail and is crying. Other members look at the floor, and one moves their chair back. What should the social worker do **FIRST**?

A. Allow the member to finish the account without interruption

B. Gently pause the member and suggest returning to it later

C. Talk privately with the member after the session ends

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Letting a member speak can feel respectful, and catharsis is a group factor. In a first session, before trust or norms exist, a detailed trauma account often leads to regret and early dropout and overwhelms other members.

B **Correct.** Group leaders protect members from disclosing more than the group's early trust can hold. A premature, intense disclosure in a first session can leave the member ashamed and push them, or others, to drop out (Yalom & Leszcz, 2020). The other members' withdrawal shows the group is not ready. Gently pausing the member, with warmth and a clear offer to return to it, protects both the member and the group (Corey, Corey & Corey, 2018). It beats A because letting the account continue honors the member's voice now at the cost of later shame and group fragmentation.

C **Incorrect** (trap: Right action, wrong time). A private check-in after the session is valuable and should follow. Waiting until then leaves the member exposed and the group overwhelmed for the rest of the disclosure.

Takeaway: In early group sessions, gently block premature intense disclosure to protect the member and the group, then return to it as trust develops.

Sources: Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, self-disclosure; premature disclosure and early dropouts; Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, leadership skills: blocking

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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51. ETH-1086 (rev 1) · Best answer: B

Which situation is an ethical dilemma rather than a clear-cut ethical violation?

A. A supervisor asks a worker to backdate a missing progress note
B. A capable client refuses a shelter bed on a freezing night
C. An agency bills insurance for canceled group sessions
D. A worker invests in a current client's small business

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Backdating creates a false record. No competing professional duty justifies it, so it is a clear violation rather than a dilemma (NASW Code 4.04).

B **Correct.** An ethical dilemma exists when two or more professional duties conflict, so honoring one compromises another (Reamer, 2018). A capable client's refusal of shelter in dangerous cold sets self-determination against protection from harm; both are legitimate duties, and the right balance depends on further facts such as risk and capacity. B beats D because investing with a client may look like a judgment call, but the Code clearly bars exploiting professional relationships for business gain (NASW Code 1.06(b)).

C **Incorrect** (trap: Wrong model or concept). Billing for sessions that did not occur is fraud, which 4.04 prohibits. Nothing about it requires weighing competing duties.

D **Incorrect** (trap: Wrong model or concept). A business relationship with a current client creates a conflict of interest and a risk of exploitation that 1.06(b) addresses directly. It is a clear boundary problem, not a conflict between duties.

Takeaway: A dilemma pits two legitimate duties against each other; a clear violation has no competing duty to weigh.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical dilemmas: conflicting duties; NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 4.04
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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52. ASM-1557 (rev 1) · Best answer: D

At a health center visit, a client says their partner always knows where they have been and quotes their private texts back to them. The client found an unfamiliar app on their phone and wants the social worker's help deleting it right away. The client adds that the partner 'explodes' when not in control. What should the social worker do **FIRST**?

A. Help the client remove the app during the visit
B. Urge the client to report the tracking to police now
C. Explain that the partner's monitoring is a form of abuse
D. Plan for safety, using a device the partner cannot see

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Removing the app may be the right step later, and it is the client's choice. Done before a plan exists, it can tip off a controlling partner and erase evidence that the monitoring happened.

B **Incorrect** (trap: Overrides the client's choice). Police involvement is the client's decision and may become part of the plan (NASW Code 1.02). Urging an immediate report overrides that choice and could raise danger if the partner learns of it before safety steps are ready.

C **Incorrect** (trap: Good but not best). Naming the behavior as abuse can validate the client's experience, but it does not address the immediate risk. The client is about to change the phone, so planning for safety is the pressing need.

D **Correct.** Technology can extend partner abuse: tracking apps let a partner follow location and messages. Survivor-safety guidance warns that some abusers escalate when monitoring stops, so safety planning comes before removal, and planning should happen on a device the partner cannot access (NNEDV Safety Net, 2022). This partner reacts explosively to losing control. It beats A because deleting the app during the visit, though it is what the client asked for, could alert the partner and destroy evidence before a plan is in place.

Takeaway: When a partner monitors a client's phone, plan for safety on a secure device before removing tracking tools, since removal can trigger escalation.

Sources: NNEDV Safety Net Project (2022). Stalkerware: Phone Surveillance & Safety for Survivors, removing stalkerware; safety planning first; NASW Code of Ethics (2021), 1.02 Self-Determination
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.21 Technology and social media effects on clients · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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53. INT-1006 (rev 3) · Best answer: C

PRETEST — unscored on this form; do not count it

A patient on an inpatient psychiatric unit is in a third admission this year for self-harm and has a diagnosis of borderline personality disorder. Between admissions, the patient attended weekly supportive therapy with a provider untrained in DBT. A standard outpatient DBT program has an opening. What is the **BEST** discharge plan?

A. Resume weekly supportive therapy with an updated safety plan
B. Step the patient down to partial hospital, then supportive therapy
C. Refer the patient for comprehensive DBT
D. Add DBT skills worksheets to the current supportive therapy

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). An updated safety plan belongs in any discharge, but returning to the same treatment that has not prevented three admissions repeats a failing plan when a stronger, evidence-based option is open.

B **Incorrect** (trap: Good but not best). A partial hospital program can ease the step down from inpatient care, and many teams consider it after repeated admissions. It is time-limited, though, and it returns the patient to the same supportive therapy that has not prevented three admissions, while an evidence-based program for recurrent self-harm is open now.

C **Correct.** Comprehensive DBT combines individual therapy, a skills group, between-session coaching, and a therapist consultation team. It was developed for chronic self-harm and has the strongest evidence for borderline personality disorder (Linehan, 1993). Three admissions despite supportive therapy show the current plan is

not enough, and a full program is available. It beats D because worksheets added to untrained supportive therapy give only one piece of DBT, without the coaching, structure, and team that make it effective.

D **Incorrect** (trap: Good but not best). Skills practice helps, but worksheets delivered by a provider untrained in DBT lack individual DBT therapy, coaching, and the consultation team. Partial DBT is a fallback only when a full program is unavailable.

Takeaway: For recurrent self-harm with borderline personality disorder, refer to comprehensive DBT when available rather than repeating a failing plan.

Sources: Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, modes of comprehensive DBT; target hierarchy
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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54. ETH-1576 (rev 2) · Best answer: B

A Black client in outpatient treatment for depression asks the white social worker about working with a Black social worker at the clinic instead. Invited to say more, the client says the work has helped but that they want to talk about racism at work without explaining it first. A Black colleague has openings. What should the social worker do **NEXT**?

A. Explore whether the request reflects discomfort with the current work
B. Arrange the transfer and offer a joint handoff session
C. Propose a few more sessions to work through the concern together
D. Reassure the client that the worker is willing to learn

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The social worker has already asked, and the client gave a clear reason tied to the burden of explaining racism. Probing again for hidden discomfort treats an informed choice as a symptom. More exploration fits when the reason is unclear.

B **Correct.** Clients have the right to make informed choices about their care, and asking for a provider who shares one's racial identity is a legitimate choice that can lift the burden of explaining racism (NASW Code 1.02; 1.05(c)). The client has explained the reason, values the work, and a colleague is available, so the next step is an orderly, warm transfer (NASW Code 1.16(b)). It beats C because pressing for more sessions puts the worker's wish for continuity ahead of a clear client choice; the joint handoff protects continuity instead.

C **Incorrect** (trap: Overrides the client's choice). Continuity matters, but the client has made an informed choice and a provider is available. Asking for more sessions first puts the worker's preference ahead of the client's. A short transition is appropriate if the client wants one.

D **Incorrect** (trap: Good but not best). Offering to learn is well meant, but it asks the client to keep doing the explaining they want to avoid. It centers the worker's growth rather than the client's stated need.

Takeaway: When a client makes an informed request for a provider who shares their identity, honor it with an orderly, warm transfer.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 1.16(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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55. ETH-1632 (rev 1) · Best answer: D

A university team plans to study blood samples that members of a tribal community gave years ago for diabetes research. The new study would analyze the samples for population ancestry. The samples are de-identified, and the university's review board has ruled that new consent is not required. The social worker on the team knows the community has had samples used in ways members never agreed to. What is the **MOST** appropriate step?

A. Proceed, as the review board has approved the plan
B. Contact the original donors to ask for individual consent
C. Share study results with tribal leaders once analysis ends
D. Seek the tribal government's approval before any reuse

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Review board approval meets the regulatory minimum, and de-identified data often fall outside consent rules. Legal permission does not settle the ethical question here, where reuse for ancestry analysis mirrors past harm and the tribe has its own authority over research on its people.

B **Incorrect** (trap: Good but not best). Asking donors is more respectful than proceeding without notice, but ancestry findings affect the whole community, not only the individual donors. Tribal government approval comes first; individual consent can follow if the tribe agrees to the study.

C **Incorrect** (trap: Right action, wrong time). Returning results to the community is good practice in community-engaged research. Sharing them only after the analysis presents a finished decision and repeats the pattern of acting first and informing the community later.

D **Correct.** Research exploitation has lasting effects on Indigenous communities. In the Havasupai case, blood given for diabetes research was later used for other studies, including ancestry work, without the tribe's agreement, and the dispute ended in a 2010 settlement (Mello & Wolf, 2010). Tribal nations are sovereign governments and may set their own research approval rules. It beats B because individual consent, though respectful, bypasses the tribe's collective authority over research about shared ancestry. The Code asks researchers to weigh consequences and follow guidelines that protect participants (NASW Code 5.02(d)).

Takeaway: When research touches a community harmed by past research exploitation, seek collective approval, such as tribal government consent, beyond minimum legal review.

Sources: Mello, M. M., & Wolf, L. E. (2010). The Havasupai Indian tribe case: Lessons for research involving stored biologic samples. New England Journal of Medicine, 363(3), 204-207, whole article; NASW Code of Ethics (2021), 5.02(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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56. INT-1569 (rev 2) · Best answer: B

A residential treatment social worker plans a point system to reduce a 15-year-old's verbal aggression on the living unit, in the classroom and during family visits. The team wants evidence that the point system, rather than outside events, produces any change. Staff will not remove the point system once it starts working. Which evaluation design is **MOST** appropriate?

A. Reversal (ABAB) design
B. Multiple-baseline design
C. AB design

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Withdrawing and then reinstating the intervention gives strong causal evidence, but it requires removing a working program. The team will not do that, so this design does not fit.

B **Correct.** A multiple-baseline design starts the intervention at staggered times across behaviors, settings or people. If aggression drops in each place only when the point system begins there, outside events become an unlikely explanation, and no withdrawal is needed (Bloom, Fischer & Orme, 2009). The three locations suit this design, and staff refuse to remove a working intervention. It beats A because a reversal design gets its causal evidence by withdrawing the intervention, which the team will not do.

C **Incorrect** (trap: Wrong model or concept). An AB design shows whether change followed the intervention but cannot rule out outside events, such as a staff change or family news, so it does not meet the team's need for causal evidence.

Takeaway: To show an intervention caused change without withdrawing it, stagger its start across settings, behaviors or people in a multiple-baseline design.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** MOST APPROPRIATE · **Cognitive** recall · **Difficulty** 1 of 3

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57. ASM-1100 (rev 2) · Best answer: B

At a family clinic, the parent of a 4-year-old says the child touches their own genitals at nap time and recently asked why a baby sibling 'looks different.' The touching stops when the parent offers a toy. The parent's voice shakes: 'I just have a bad feeling something happened.' What should the social worker do **FIRST**?

A. Make a report of possible sexual abuse
B. Ask the parent what has made them worried
C. Ask the child directly whether anyone has touched them
D. Explain to the parent that this is typical curiosity for the age

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A report rests on reasonable suspicion, such as a disclosure, sexual knowledge beyond the child's age or coercive acts with other children. Self-touch that stops with simple redirection does not reach that threshold, though what the parent says next might. Reporting now would start an investigation on facts that do not yet point to abuse.

B **Correct.** Touching one's own genitals and curiosity about others' bodies are common in preschoolers; concern rises when behavior goes beyond the child's stage, persists despite redirection or is coercive (NCTSN, 2009). The behaviors described are typical, but the parent's sense that 'something happened' may rest on facts not yet shared, such as a new caregiver, a remark by the child or a change in mood. Exploring the worry first respects the parent and gathers the facts that decide between reassurance and a report. It beats D because reassuring before hearing what drives the worry could miss a real warning sign.

C **Incorrect** (trap: Outside the social work role). Questions about abuse belong with trained interviewers once indicators are present, and they should be open-ended. Asking a 4-year-old a pointed question now risks leading the child, adding fear and weakening any later forensic interview.

D **Incorrect** (trap: Right action, wrong time). The behaviors described are typical for a 4-year-old, so this explanation will likely be part of the answer. Offered before the social worker learns what lies behind the parent's 'bad feeling,' it may dismiss information that matters.

Takeaway: When a parent fears abuse on the basis of age-typical behavior, first learn what drives the worry; then reassure or report based on the facts.

Sources: National Child Traumatic Stress Network (2009). Sexual Development and Behavior in Children: Information for Parents and Caregivers, Common sexual behaviors; when to be concerned
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

58. ETH-1568 (rev 1) · Best answer: B

According to the NASW Code of Ethics, what should field instructors ensure when social work students provide services to clients?

A. Students' work is reviewed before each session begins
B. Clients are routinely told that a student is serving them
C. Clients sign a separate consent form for student services

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Field instructors review students' work regularly, but the Code sets no rule requiring review before every session. The standard here concerns the client's right to know who is serving them.

B **Correct.** Educators and field instructors take reasonable steps to ensure that clients are routinely informed when services are provided by students (NASW Code 3.02(c)). Clients have a right to know who is serving them and in what role, which supports informed consent (1.03(a)). It beats C because the Code requires that clients be informed, not that a separate signed consent be obtained; an agency may add a form, but informing clients is the ethical standard.

C **Incorrect** (trap: Misreads law or ethics). Some agencies use written acknowledgment forms, but the Code's standard is that clients are routinely informed. A separate consent form is a local practice, not the ethical requirement.

Takeaway: Clients should be routinely told when a social work student is providing their services.

Sources: NASW Code of Ethics (2021), 3.02(c); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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59. INT-1614 (rev 1) · Best answer: A

Six months ago, an outpatient clinic adopted a policy requiring a suicide risk screen at every intake, with a same-day safety plan for positive screens. All clinicians signed the policy and finished training. The quality committee now wants to know whether the policy is reducing risk. What should the committee do **FIRST**?

A. Audit a sample of charts for completed steps
B. Compare suicide attempts among clients before and after adoption
C. Survey clinicians about how useful they find the screen
D. Report the signature and training totals to the board

Why each option is right or wrong

A **Correct.** Evaluating a risk-reducing policy starts by checking whether it is actually followed, since outcomes cannot be credited to a policy that is not in use. Reamer's ethics audit, for example, compares written policy with what staff actually do (Reamer, 2001). Signatures and training show only that staff were informed; a record audit shows whether screens and same-day safety plans occur. It beats B because outcome comparisons come after fidelity is established, and suicide attempts are infrequent enough that 6 months of data would be hard to interpret.

B **Incorrect** (trap: Right action, wrong time). Outcome data matter eventually, but a before-and-after comparison means little until the committee knows the policy is being carried out. Suicide attempts are also infrequent, so 6 months of data would be hard to interpret.

C **Incorrect** (trap: Good but not best). Clinician views can reveal barriers, but usefulness ratings do not show whether screens and safety plans actually happen. A survey can supplement a record audit.

D **Incorrect** (trap: Misuses the technique). Signature and training totals show that staff were informed, not that practice changed. Reporting them as results would overstate what is known.

Takeaway: Evaluate a risk policy by first confirming it is followed in practice; awareness measures and outcome comparisons come later or fall short.

Sources: Reamer, F. G. (2001). The Social Work Ethics Audit: A Risk Management Tool. NASW Press, Audit steps and risk ratings

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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60. ASM-1500 (rev 1) · Best answer: B

A hospital social worker meets with a 5-year-old whose grandparent had a stroke this morning. The child says the stroke happened because the child refused to share toys at breakfast. Later, the child pats the hospital bed and says the bed feels sad. According to Piaget, this thinking is **MOST** likely typical of which stage?

A. Sensorimotor

B. Preoperational

C. Concrete operational

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The sensorimotor stage covers roughly birth to age 2, when infants learn through senses and movement and develop object permanence. A 5-year-old who builds a verbal story about cause and effect has moved past it into symbolic thought.

B **Correct.** In Piaget's preoperational stage (about ages 2 to 7), children think symbolically but egocentrically, often using magical thinking and animism (Piaget & Inhelder, 1969). This 5-year-old believes a personal act caused the grandparent's stroke and gives feelings to a bed, both hallmarks of preoperational thought. It beats C because concrete operational children, from about age 7, reason logically about physical causes and usually understand that their behavior cannot cause another person's illness. Recognizing this helps the social worker correct the child's self-blame in simple, concrete words.

C **Incorrect** (trap: Wrong model or concept). Concrete operational thinking begins around age 7 and brings logical reasoning about real objects and events, including conservation. Children at that stage usually recognize that refusing to share cannot cause a stroke, so this child's self-blame and animism point to an earlier stage.

Takeaway: Self-blame through magical thinking and giving feelings to objects mark Piaget's preoperational stage, roughly ages 2 to 7.

Sources: Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, preoperational thought: egocentrism and animism
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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61. ASM-1505 (rev 1) · Best answer: C

A 19-year-old at a university counseling center is failing pre-nursing courses. Both parents are nurses, and the client says the plan to become a nurse was set at age 12. Asked what other fields have interested them, the client looks puzzled and says they have never looked into anything else. Which identity status does this **MOST** likely reflect?

A. Identity moratorium
B. Identity achievement
C. Identity foreclosure
D. Identity diffusion

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Moratorium is active exploration without a firm commitment yet. This client is committed to nursing and has not explored alternatives, which is the reverse pattern.

B **Incorrect** (trap: Wrong model or concept). Identity achievement follows real exploration that ends in a chosen commitment. The client's firm commitment makes this tempting, but it was adopted from the parents at age 12 without examining other options.

C **Correct.** Marcia's foreclosure status describes a firm commitment made without exploring alternatives, often adopted from parents (Marcia, 1966). This client committed to nursing in childhood, following both parents, and has never considered other paths, even while failing the coursework. It beats B because identity achievement requires a period of active exploration before commitment; this client has the commitment but skipped the exploration. The academic setback may open a healthy period of exploration that the social worker can support.

D **Incorrect** (trap: Wrong model or concept). Diffusion means neither exploration nor commitment, often with drifting or apathy. This client is strongly committed to one path, so diffusion does not fit.

Takeaway: Foreclosure is commitment without exploration, often adopted from parents; achievement is commitment after exploration; moratorium is exploration without commitment.

Sources: Marcia, J. E. (1966). Development and validation of ego-identity status. Journal of Personality and Social Psychology, 3(5), 551-558, identity statuses
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

Section 2

62. ETH-1551 (rev 1) · Best answer: D

During weekly chart review, a clinical supervisor at a community agency reads a supervisee's note from 4 days ago. The client said, 'Sometimes I think everyone would be better off without me.' The note shows no follow-up questions, and the client's next appointment is in 10 days. What should the supervisor do **FIRST**?

A. Ask the supervisee why the remark went unexplored
B. Arrange suicide risk assessment training for the supervisee
C. Note the lapse in the supervisee's performance record
D. Make sure the client is contacted today to assess risk

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Talking with the supervisee about the missed follow-up is an important step and part of the teaching role. Doing it first delays reaching a client whose possible suicidal statement has gone unassessed for 4 days.

B **Incorrect** (trap: Right action, wrong time). Training addresses the skill gap and protects future clients, so it belongs in the plan. It does nothing for this client, who remains unassessed with no contact scheduled for 10 days.

C **Incorrect** (trap: Right action, wrong time). Supervision records should capture the issue and the response, but recording it first puts paperwork ahead of the client. The supervision standards also treat errors as chances to learn, not grounds for punishment (Supervision Standards, Evaluation and Outcomes).

D **Correct.** Supervision exists to protect clients, and supervisors are responsible for ensuring that supervisees provide competent services (Supervision Standards, Overview). The client's remark may signal suicidal thinking, it was never assessed, and 10 days remain before the next contact. Arranging same-day outreach so risk is assessed comes before the supervisee's learning needs, because a social worker's primary duty is the client's well-being (NASW Code 1.01). It beats A because understanding the lapse matters and should follow within the day, but that talk can wait a few hours; an unassessed risk cannot.

Takeaway: When a supervisor finds an unaddressed client risk, protecting the client comes first; teaching and documenting the lapse follow.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Evaluation and Outcomes; NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

63. ASM-1015 (rev 3) · Best answer: D

A hospital social worker meets with a 40-year-old patient admitted with sudden weakness in the left leg after a car accident that caused no injury. The medical team's exam findings do not match any known nerve or brain disease, and imaging is normal. The patient is upset about missing work. What is the **MOST** likely diagnosis?

A. Malingering
B. Factitious disorder imposed on self
C. Somatic symptom disorder
D. Functional neurological symptom disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Malingering is intentional faking for an external reward, such as avoiding work or gaining money. Nothing here suggests deception, and the patient wants to get back to work.

B **Incorrect** (trap: Near-miss diagnosis). Factitious disorder involves deliberately falsifying or causing illness to be seen as sick, without an outside reward. The facts give no sign of deception, which this diagnosis requires.

C **Incorrect** (trap: Near-miss diagnosis). Somatic symptom disorder is a close neighbor in the same chapter, but it centers on excessive, persistent thoughts, feelings or behaviors about bodily symptoms, such as constant worry or repeated care-seeking. This patient shows only proportionate concern about missing work. The defining finding is a motor deficit incompatible with known neurological disease.

D **Correct.** Functional neurological symptom disorder (conversion disorder) involves altered movement or sensation, with clinical findings showing the problem is incompatible with recognized neurological or medical disease (DSM-5-TR). Sudden leg weakness with exam findings that fit no nerve or brain disease and normal imaging fits. A stressor such as the accident is common but not required. It beats A because malingering involves intentionally producing symptoms for an outside gain; nothing suggests deliberate production, and the patient is eager to return to work.

Takeaway: Motor or sensory loss inconsistent with known disease, without intentional production, suggests functional neurological symptom disorder.

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Functional Neurological Symptom Disorder (Conversion Disorder)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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64. INT-1159 (rev 1) · Best answer: B

Who studied survivors of the Cocoanut Grove nightclub fire and described acute grief as a normal reaction that runs its course?

A. Gerald Caplan
B. Erich Lindemann
C. Elisabeth Kubler-Ross
D. Naomi Golan

Why each option is right or wrong

- **A** **Incorrect** (trap: Wrong model or concept). Caplan worked alongside Lindemann and is credited with defining crisis as a temporary state of disequilibrium when usual coping fails, and with preventive psychiatry. The fire study itself was Lindemann's.
- **B** **Correct.** Erich Lindemann studied survivors and relatives of those who died in the 1942 Cocoanut Grove fire in Boston. He described acute grief as a normal syndrome with a predictable course that brief help could support, and this work became a foundation of crisis theory (Lindemann, 1944). It beats A, Gerald Caplan, because Caplan built on Lindemann's work to define crisis as disequilibrium when usual coping fails; he did not conduct the Cocoanut Grove study.
- **C** **Incorrect** (trap: Wrong model or concept). Kubler-Ross described reactions to dying, drawn from interviews with terminally ill patients in the 1960s. Her work did not come from the Cocoanut Grove survivors.
- **D** **Incorrect** (trap: Wrong model or concept). Golan developed crisis concepts such as the hazardous event and the precipitating factor, decades after the fire. She did not study its survivors.

Takeaway: Lindemann's Cocoanut Grove study framed acute grief as normal and time-limited; Caplan later defined crisis as disequilibrium when coping fails.

Sources: Lindemann, E. (1944). Symptomatology and management of acute grief. American Journal of Psychiatry, 101(2), 141-148, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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65. ETH-1012 (rev 2) · Best answer: B

A social worker meets weekly with a 15-year-old client in outpatient therapy. At intake, the parent agreed that session details would stay private unless a safety concern arose. A month later the parent calls, worried, and asks what the teen has been talking about. What is the **MOST** appropriate response?

- **A.** Tell the parent nothing can be shared without the teen's consent
- **B.** Give general progress and revisit the confidentiality agreement
- **C.** Suggest that the parent ask the teen directly

Why each option is right or wrong

- **A** **Incorrect** (trap: Misreads law or ethics). Protecting the teen's privacy is right, which makes this tempting. The intake agreement kept session details private, not all information, so a flat refusal shuts out a worried parent, overstates the agreement and may weaken the parent's support for treatment.
- **B** **Correct.** When a parent agrees to a confidential relationship between a provider and a minor, HIPAA does not treat the parent as the minor's personal representative for that care (45 CFR 164.502(g)(3)(i)(C)). The Code calls for discussing the limits of confidentiality early and as needed (NASW Code 1.07(e)). Sharing general progress and revisiting the agreement respects the parent's concern while protecting the teen's trust. B beats C because redirecting the parent is reasonable but leaves the worry unanswered and the agreement unexplained.

C **Incorrect** (trap: Good but not best). Encouraging parent-teen communication is useful, but by itself it sidesteps the parent's worry. The social worker can offer general progress and restate the agreement, and may also suggest a family conversation.

Takeaway: Honor a confidentiality agreement made with a minor and parent at intake; offer general progress and restate its limits.

Sources: HIPAA Privacy Rule, 45 CFR 164.502(g)(3)(i)(C); NASW Code of Ethics (2021), 1.07(e)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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66. ETH-1665 (rev 1) · Best answer: C

A social worker begins voluntary in-home parenting support with a family. At each visit, the parent sets out tea and a small plate of homemade pastries, explaining that in their culture a guest is always served. The family has offered nothing else. What is the **MOST** appropriate response?

A. Decline politely, explaining that workers cannot accept gifts
B. Accept the tea and bring a small treat to the next visit
C. Accept the refreshments graciously as hospitality

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A blanket refusal misreads hospitality as a gift that creates obligation. Declining makes sense for items of real value or offers tied to a request, but here it would likely offend the parent and strain the work.

B **Incorrect** (trap: Boundary slip or collusion). Returning the favor turns a guest courtesy into an exchange of gifts and moves the relationship toward the social. The worker can accept hospitality without reciprocating; warm thanks is enough.

C **Correct.** The Code has no specific gift standard; gifts and hospitality are weighed under its boundary standards by value, motive, timing and cultural meaning (NASW Code 1.06(c)). Simple refreshments offered as customary hospitality are a minor boundary crossing that honors the family's culture, and refusing can shame the host and harm engagement (Reamer, 2020). The offer is what any guest receives, and nothing else is given. It beats A because declining treats a cultural courtesy as a gift that creates obligation, when nothing suggests any expectation.

Takeaway: Customary hospitality of small value, offered as a cultural courtesy, can be accepted; weigh value, motive and meaning.

Sources: NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts, favors and home visits
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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67. ETH-1138 (rev 1) · Best answer: C

A child welfare court orders a parenting capacity assessment, and a social worker is assigned to complete it. At the first meeting, the parent says, 'I'm so glad I finally have someone to talk to about everything.' What should the social worker do **FIRST**?

A. Begin the interview while the parent is open and engaged
B. Offer supportive counseling sessions alongside the assessment
C. Explain the evaluator role and who will receive the report

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Interviewing is the core of the assessment and follows soon. Starting while the parent believes the worker is a confidential helper means they share information without knowing it will go to the court.

B **Incorrect** (trap: Boundary slip or collusion). Combining evaluator and counselor roles creates a dual relationship with a risk of harm, since the worker would judge the same person they are helping (1.06(c)). A referral for separate counseling is the better route if the parent wants support.

C **Correct.** When services are involuntary, clients are told what the service involves and how far they can refuse it, and consent rests on a clear explanation of purpose and limits (NASW Code 1.03(a), 1.03(d)). The parent's comment shows they expect a confidential helping relationship, but a court-ordered assessment goes to the court and the child welfare agency. Clarifying the role, the report's recipients and the limits of confidentiality comes before any interviewing. C beats A because information gathered while the parent misunderstands the role is not given with informed consent.

Takeaway: In court-ordered evaluations, explain the role, report recipients and confidentiality limits before gathering any information.

Sources: NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(d); NASW Code of Ethics (2021), 1.06(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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68. ASM-1567 (rev 1) · Best answer: A

A client with limited English arrives for intake at an outpatient substance use program. The scheduled interpreter is trained and certified. While the interpreter steps out, the client tells the social worker in basic English, 'That interpreter knows my family. Our community is small.' When the interview resumes, the client gives short answers whenever drinking comes up. What is the **BEST** action?

A. Switch to a remote interpreter with no local ties
B. Remind the client that interpreters keep information private
C. Continue the intake in English using simpler questions
D. Ask the interpreter privately whether they know the client

Why each option is right or wrong

A **Correct.** Interpreted interviews work only if the client can speak freely. Interpreters are bound to confidentiality and impartiality (NCIHC, 2004), yet a client in a small language community may still fear that information will reach family, and here that fear is shutting down disclosure about drinking. A remote interpreter with no ties to the local community keeps professional interpretation while removing the conflict (NASW Cultural Competence Standard 9). It beats B because a reminder about the rules does not resolve the client's concern about a personal connection.

B **Incorrect** (trap: Good but not best). Interpreters are bound to confidentiality, so the reminder is accurate. But the client named a personal link to the interpreter and is already holding back; a reminder leaves the source of discomfort in the room.

C **Incorrect** (trap: Misuses the technique). The client's English is limited, so continuing in English risks misunderstanding on sensitive topics such as substance use. Language access standards call for a qualified interpreter, not a workaround.

D **Incorrect** (trap: Misreads law or ethics). Checking with the interpreter may seem fair, but it reveals what the client shared in confidence and turns the client's concern into a question for the interpreter instead of honoring the client's comfort.

Takeaway: When a client fears an interpreter's community ties, arrange a qualified interpreter from outside that community to protect open disclosure.

Sources: National Council on Interpreting in Health Care (NCIHC) (2004). National Code of Ethics for Interpreters in Health Care, Confidentiality; Impartiality; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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69. ETH-1539 (rev 3) · Best answer: B

A client with schizophrenia lives in a Housing First apartment program. The client wants to stop attending the program's weekly skills group and use that time at a peer-run drop-in center, explaining that peer staff there helped them find a part-time job and that the extra hours would help them keep it. A case manager warns that skipping the group could put the client's apartment at risk. Which response by the social worker is **MOST** helpful?

A. Encourage attending both until the client seems more stable

B. Support the change and revise the plan together

C. Explain that the apartment depends on attending the group

D. Explore what the client hopes to gain at the drop-in center

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Asking the client to do both until they seem more stable keeps a service the client wants to drop and sets a vague condition the client cannot meet. Nothing in the situation suggests a risk that would justify overriding the choice.

B **Correct.** In Housing First, housing is not contingent on treatment participation, and tenants choose which services to use (Tsemberis, 2010). Self-determination also covers refusing one service and choosing another (NASW Code 1.02). The client has named a

specific alternative that has already supported recovery and a concrete goal for the time (keeping the new job), so the social worker backs the change and revises the plan. It beats D because the client has already said what the center offers and what they hope to gain. It beats C because tying the apartment to group attendance contradicts the program model; the case manager's concern needs correcting.

C **Incorrect** (trap: Misreads law or ethics). This repeats the case manager's misunderstanding. In a Housing First program, tenancy rests on ordinary lease obligations, not group attendance. Presenting attendance as a housing condition pressures the client with an inaccurate threat.

D **Incorrect** (trap: Good but not best). Exploring a client's hopes is useful when the reasons are unclear. Here the client has already said what the center offers and what the extra hours are for, keeping the new job, so more exploration repeats ground already covered and delays acting on a clear, reasonable choice.

Takeaway: Clients may refuse one service and choose another; in Housing First, housing does not depend on treatment participation.

Sources: Tsemberis, S. (2010). Housing First: The Pathways Model to End Homelessness for People with Mental Illness and Addiction. Hazelden, separating housing from treatment; consumer choice; NASW Code of Ethics (2021), 1.02

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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70. ASM-1028 (rev 1) · Best answer: C

During a telehealth intake, a 27-year-old client describes 3 years of shifting moods. Every few weeks the client has 2 or 3 days of extra energy, talkativeness and less need for sleep, followed by a week or so of low mood, tiredness and poor concentration. The client has kept the same job throughout and has rarely felt steady for more than a few weeks at a time. Which diagnosis is **MOST** likely?

A. Bipolar II disorder
B. Persistent depressive disorder
C. Cyclothymic disorder
D. Borderline personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar II needs at least one hypomanic episode of 4 or more days and one major depressive episode of 2 or more weeks. These highs and lows are too brief to meet either threshold.

B **Incorrect** (trap: Near-miss diagnosis). Persistent depressive disorder covers chronic low mood for 2 years, but it is not diagnosed when there are recurring periods of hypomanic symptoms. The energetic, talkative stretches point to the bipolar spectrum.

C **Correct.** Cyclothymic disorder involves at least 2 years, in adults, of many periods with hypomanic symptoms and depressive symptoms that never meet full episode criteria, with no symptom-free stretch longer than 2 months (DSM-5-TR). Highs of 2 or 3 days fall short of the 4 days a hypomanic episode needs, and lows of about a week fall short of a major depressive episode. It beats A because bipolar II requires at least one full

hypomanic episode and one major depressive episode, which these brief periods do not reach.

D **Incorrect** (trap: Near-miss diagnosis). Borderline personality disorder brings mood shifts lasting hours, usually after interpersonal stress, along with unstable relationships and self-image. Mood periods lasting days, alternating up and down, fit a mood disorder better.

Takeaway: Two or more years of repeated sub-threshold highs and lows, never a full episode, is cyclothymic disorder; full episodes mean bipolar I or II.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Cyclothymic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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71. ETH-1066 (rev 1) · Best answer: C

A client with low vision asks the agency to email a copy of their record to a personal email account. The agency usually uses an encrypted portal, but the client finds it hard to use with screen-reading software. What is the **BEST** response?

A. Require the client to use the encrypted portal for access
B. Mail a printed copy to the client's home address instead
C. Warn of email risks, then send it if the client accepts
D. Email a brief summary rather than the complete record

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Encryption is the right default (1.07(m)), but once warned, a client may choose regular email for their own records. Insisting on a portal the client's screen reader cannot use blocks access.

B **Incorrect** (trap: Good but not best). A printed copy is hard for a client with low vision to read and ignores the format requested. Paper is reasonable only if the client prefers it.

C **Correct.** Clients have a right to reasonable access to their records, and agencies inform clients how technology is used to provide that access (NASW Code 1.08(a), 1.08(b)). Federal guidance says a client may receive records by unencrypted email on request after being warned that others could read them in transit (HHS FAQ 2060). The portal is a barrier for this client, so honoring the request after a clear warning respects both access and informed choice. C beats A because a security preference cannot deny a client's chosen, lawful form of access.

D **Incorrect** (trap: Good but not best). Clients are entitled to their records, not a summary they did not request (1.08(a)). A summary may be offered in addition, but it cannot replace the copy asked for.

Takeaway: After warning about risks, honor a client's request to receive their own records by regular email; security defaults cannot bar chosen access.

Sources: NASW Code of Ethics (2021), 1.08(a); NASW Code of Ethics (2021), 1.08(b); NASW Code of Ethics (2021), 1.07(m); HHS Office for Civil Rights, HIPAA FAQ, FAQ 2060: transmitting PHI to an individual by an unsecure method on request
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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72. ETH-1083 (rev 1) · Best answer: A

A community mental health agency accepts a state grant that requires entering each client's name and diagnosis into a shared state database. The social worker believes this conflicts with clients' privacy and raises it with the supervisor. The supervisor agrees but says the program cannot change the grant terms. Enrollment begins next month. What should the social worker do **NEXT**?

A. Ask the ethics committee to review the requirement
B. Let each client decide whether to be entered
C. Enter client data as the grant requires
D. Leave client names out of the database entries

Why each option is right or wrong

A **Correct.** When agency policy and the Code conflict, social workers make a good-faith effort to resolve it; if no reasonable resolution appears possible, they seek proper consultation, such as an ethics committee or legal counsel, before deciding (NASW Code, Purpose). The supervisor step has failed and a month remains, so formal consultation is next. A beats B because clients must learn about the database at consent (1.03(a)), but an opt-out the grant does not allow would be a promise the agency cannot keep.

B **Incorrect** (trap: Misreads law or ethics). Clients do need to hear about the database before services begin (1.03(a)). Offering each one a choice the grant terms do not permit misleads them and leaves the conflict unresolved.

C **Incorrect** (trap: Right action, wrong time). Complying may turn out to be the outcome, but doing it before consultation treats the funder's rule as settling the ethical question. Employer policies should not override ethical practice without that review (3.09(d)).

D **Incorrect** (trap: Misreads law or ethics). Quietly altering required entries is covert noncompliance that could put funding and services at risk and breaks commitments to the employer (3.09(a)). Disagreement belongs in consultation, not hidden action.

Takeaway: When agency policy conflicts with ethics and the supervisor cannot resolve it, seek formal consultation before deciding how to act.

Sources: NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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73. INT-1682 (rev 1) · Best answer: A

A first-year school social worker co-leads a social skills group for 8-year-olds. Sessions keep unraveling when two members begin taunting each other, and the social worker freezes. Discussing session notes in weekly supervision has produced sound plans that fall apart in the moment. Which supervision format is **BEST** suited to this need?

A. Live supervision with in-session coaching
B. Review of session recordings afterward
C. Peer consultation with other school staff
D. Group supervision with case presentations

Why each option is right or wrong

A **Correct.** Live supervision lets the supervisor observe a session directly, through a one-way mirror or by video, and coach the worker as events unfold, for example with a bug-in-the-ear device or a brief consultation break (Bernard & Goodyear, 2019). This worker's plans are sound but collapse in the moment, so feedback has to arrive during the session. It beats B because recordings show accurately what happened but still deliver feedback after the moment has passed, the same gap note-based supervision has.

B **Incorrect** (trap: Wrong model or concept). Recordings improve on self-report by showing what actually happened, and they help when the problem is unclear. The difficulty here is applying sound plans in real time, which after-the-fact review cannot coach.

C **Incorrect** (trap: Wrong model or concept). Peer consultation offers ideas and support but carries no supervisory role and still happens outside the session. It would not give a beginning worker real-time guidance.

D **Incorrect** (trap: Wrong model or concept). Group supervision builds shared learning and normalizes struggles, but it relies on case presentations after the fact. It suits a need several workers share better than one worker's in-the-moment difficulty.

Takeaway: When a supervisee's plans are sound but break down during sessions, live supervision provides feedback in real time.

Sources: Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Live supervision

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 2 of 3

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74. INT-1079 (rev 1) · Best answer: A

On a text-based crisis line, a person writes that they swallowed a full bottle of a prescription sleep medication 30 minutes ago and now feel 'calm and ready.' Earlier in the chat they gave their home address. When the social worker raises getting help, the person replies, 'Please don't send anyone.' What should the social worker do **FIRST**?

A. Send emergency medical services to the person now
B. Ask what happened today that led to taking the pills
C. Work with the person on a safety plan for tonight

Why each option is right or wrong

A **Correct.** Lethality and imminent danger come first in crisis intervention (Roberts & Ottens, 2005), and a person who has already swallowed a full bottle of medication is in a medical emergency. Social workers may limit self-determination when a client's actions pose a serious, foreseeable and imminent risk to self (NASW Code 1.02). The worker has an address and can send help while continuing to text. It beats B because understanding the trigger matters, but every minute spent exploring delays treatment for a possibly fatal overdose; the conversation can continue while help is on the way.

B **Incorrect** (trap: Right action, wrong time). Exploring the precipitating event is part of crisis work once immediate danger is handled. Here an overdose is already in progress, and asking about the day's events delays the medical help the person needs now.

C **Incorrect** (trap: Ignores risk). Safety planning helps prevent a future attempt. It cannot reverse pills already swallowed, and the 'calm' the person describes may reflect the medication or a decision already made. Medical care comes first; a safety plan can follow recovery.

Takeaway: When a person has already acted on a suicide attempt, emergency medical help comes before exploring causes or planning, even over objection.

Sources: Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 1: lethality and imminent danger assessment; NASW Code of Ethics (2021), 1.02
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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75. ASM-1608 (rev 1) · Best answer: D

The ASAM Criteria (4th edition) rate a person's needs on six dimensions. Which dimension covers how safe and supportive the person's home is, such as roommates who use?

A. Substance use-related risks
B. Person-centered considerations
C. Intoxication, withdrawal, and addiction medications
D. Recovery environment interactions

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Dimension 4 rates the person's own likelihood of continued use and its related harms, which may rise when others at home use. The question asks about the living situation itself, which belongs to a different dimension.

B **Incorrect** (trap: Wrong model or concept). Dimension 6, person-centered considerations, covers the person's preferences, barriers to care and readiness, used in shared decision-making about placement. The safety and support of the home is a separate dimension.

C **Incorrect** (trap: Wrong model or concept). Dimension 1 covers intoxication, withdrawal risk and the need for addiction medications. Substance use by others in the home raises relapse risk but says nothing about the person's current withdrawal state.

D **Correct.** In the ASAM Criteria (4th ed.), Dimension 5, recovery environment interactions, covers how well the person functions in the current living setting, how

safe and supportive it is and how the people in it view substance use (ASAM, 2023). Returning to a home where others use is a classic Dimension 5 concern. It beats A because Dimension 4, substance use-related risks, rates the person's own likelihood of continued use and harm, not the setting itself.

Takeaway: ASAM placement is multidimensional; the safety and support of the home, including others' substance use, fall under the recovery environment dimension.

Sources: ASAM (2023). The ASAM Criteria (4th ed.), Dimension 5: Recovery Environment Interactions
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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76. ASM-1559 (rev 1) · Best answer: C

During a home visit, a social worker sees a 2-year-old pick paint chips from a peeling windowsill and put them in their mouth. The family rents a house built in 1950. The parent says the child has been more irritable lately, sleeps poorly, and often has stomachaches. What should the social worker do **FIRST**?

A. Teach the parent calming routines for the irritability
B. Ask the landlord to repaint the windowsill
C. Help the parent arrange a blood lead test with the pediatrician

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Calming routines may help irritability from many causes, but they treat a symptom while possible lead exposure continues. Behavioral support fits after a medical cause has been checked.

B **Incorrect** (trap: Right action, wrong time). The hazard needs correcting, and renters can ask a landlord for a lead inspection. Testing the child comes first, though, because exposure may already have caused harm that needs medical follow-up.

C **Correct.** Peeling paint in homes built before 1978 is a major source of childhood lead exposure, and CDC states that no safe blood lead level in children has been identified. Even low levels are linked to learning and behavior problems. A blood lead test is the best way to learn whether this child has been exposed, so the social worker links the family to the pediatrician (CDC, Childhood Lead Poisoning Prevention). It beats B because fixing the windowsill matters but does not show whether the child already needs treatment.

Takeaway: When a young child may be eating old paint, arrange blood lead testing first; no safe blood lead level in children has been identified.

Sources: CDC (Centers for Disease Control and Prevention) (2025). About Childhood Lead Poisoning Prevention, sources (pre-1978 paint); no safe blood lead level; blood lead testing
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.22 Urbanization, globalization, environmental hazards, climate change impacts · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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77. ASM-1592 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A 14-year-old in kinship care has lived through a parent's incarceration and three school moves. The teen earns steady grades, plays in the school band and talks about future plans. Since age 8, the same basketball coach has checked in weekly and attended the teen's court hearings. Which factor **MOST** likely accounts for the teen's resilience?

A. A committed relationship with a caring adult
B. Above-average intelligence and school ability
C. Taking part in structured after-school activities

Why each option is right or wrong

A **Correct.** Research on resilience finds that children who do well despite serious hardship have had at least one stable, committed relationship with a supportive adult (National Scientific Council on the Developing Child, 2015). The coach's steady presence for 6 years, including at court hearings, is the clearest protective factor in the case. It beats C because band activity helps, but the coach's long, reliable involvement is the stronger and better-documented support.

B **Incorrect** (trap: Wrong model or concept). Cognitive ability can be protective, but the stem describes steady, not exceptional, grades. Nothing shows above-average intelligence, so this explanation rests on an assumption.

C **Incorrect** (trap: Wrong model or concept). Structured activities such as band can build skills and belonging, and they support resilience. They are less central than a long, committed relationship with a caring adult, which the case documents for 6 years.

Takeaway: A stable, committed relationship with at least one supportive adult is the most consistent protective factor for children facing adversity.

Sources: National Scientific Council on the Developing Child (2015). Supportive Relationships and Active Skill-Building Strengthen the Foundations of Resilience: Working Paper No. 13. Center on the Developing Child at Harvard University, whole paper
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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78. INT-1055 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

In emotionally focused couples therapy, during which stage does the social worker help partners see their pattern of pursuing and withdrawing as a shared enemy rather than either partner's fault?

A. Cycle de-escalation
B. Restructuring interactions
C. Consolidation

Why each option is right or wrong

A **Correct.** Emotionally focused therapy moves through three stages. In the first, cycle de-escalation, the worker tracks the negative interaction pattern, reaches the attachment fears beneath it, and frames the pattern itself as the problem both partners face. This lowers blame and makes safer contact possible (Johnson, 2019). It beats B because restructuring comes next: once the pattern is named and calmer, the withdrawn partner re-engages and the pursuing partner softens, creating new bonding events.

B **Incorrect** (trap: Wrong model or concept). Restructuring is the second stage. It builds on a calmer, already-named pattern by helping the withdrawer re-engage and the pursuer soften. Framing the pattern as a shared enemy is the groundwork laid before this stage begins.

C **Incorrect** (trap: Wrong model or concept). Consolidation is the final stage, when partners apply their new ways of relating to old problems and build a shared story of how they changed. It comes well after the pursue-withdraw pattern is first named.

Takeaway: EFT stage one de-escalates the negative cycle by framing it as the couple's common enemy; restructuring and consolidation follow.

Sources: Johnson, S. M. (2019). The Practice of Emotionally Focused Couple Therapy: Creating Connection (3rd ed.). Routledge, Stage 1: cycle de-escalation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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79. ASM-1057 (rev 2) · Best answer: C

An adult walks into an outpatient clinic asking for help staying sober. The client drank about a fifth of liquor daily for years and stopped on their own 30 hours ago. The client's hands shake, they are sweating and restless, and they briefly saw insects on the wall this morning. What should the social worker do **FIRST**?

A. Help the client enroll in outpatient withdrawal care tomorrow

B. Book an intake for the intensive outpatient program

C. Arrange same-day medical care for alcohol withdrawal

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Ambulatory withdrawal management suits many people with mild to moderate withdrawal, so this is tempting. Hallucinations 30 hours after heavy daily drinking signal more serious withdrawal during the peak seizure window, so medical evaluation cannot wait until tomorrow (ASAM, 2020).

B **Incorrect** (trap: Right action, wrong time). Intensive outpatient care may suit the client after withdrawal is managed. Booking it now leaves active withdrawal with hallucinations untreated during the period of highest seizure risk.

C **Correct.** Withdrawal after heavy daily drinking can progress to seizures and delirium tremens, which can be fatal, so it needs medical management. Tremor, sweating and restlessness 30 hours after the last drink, plus brief hallucinations, show withdrawal is under way in the window when seizures most often occur. Same-day medical care comes before further screening or program planning. It beats A because starting care tomorrow leaves the client untreated through the hours when seizures are most likely,

and hallucinations call for a medical judgment about the safe level of care now (ASAM, 2020; DSM-5-TR).

Takeaway: Visible alcohol withdrawal after heavy daily drinking needs same-day medical management before screening or program placement.

Sources: ASAM (2020). The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. Journal of Addiction Medicine, 14(3S), 1-72, Assessment and level of care; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Withdrawal

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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80. INT-1021 (rev 1) · Best answer: A

After a layoff, a client at a community job center tells the social worker they lie awake panicking about a utility shutoff notice due in 10 days. The client has not yet contacted the utility or applied for bill assistance. Which coping skill is **BEST** for the social worker to teach?

A. Break the problem into concrete steps to act on

B. Practice slow breathing when panic rises at night

C. Write about the worries in a journal before bed

Why each option is right or wrong

A **Correct.** Coping works best when it fits the stressor. When a stressor can be changed, problem-focused coping (defining the problem, generating options, and acting) reduces the threat itself; emotion-focused coping suits stressors that cannot be changed (Lazarus & Folkman, 1984). The shutoff is controllable, with 10 days left and untried options such as calling the utility and applying for help, so structured problem solving fits (D'Zurilla & Nezu, 2007). It beats B because breathing calms the nights but leaves the shutoff unaddressed, so the panic will return.

B **Incorrect** (trap: Good but not best). Slow breathing is a useful skill for nighttime panic and could be added. Used alone, it soothes the feeling while the controllable problem driving it moves closer to its deadline.

C **Incorrect** (trap: Good but not best). Journaling can reduce rumination and help sleep. Like breathing, it manages the emotion but leaves an actionable, time-limited problem untouched.

Takeaway: Match coping to the stressor: problem-focused coping for changeable problems, emotion-focused coping for stressors that cannot be changed.

Sources: Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, problem-focused and emotion-focused coping; D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, problem-solving steps

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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81. ASM-1586 (rev 1) · Best answer: C

In a primary care clinic, a client with a slow-healing foot ulcer smokes a pack a day. Asked to rate how important quitting is on a 0-10 scale, the client says 9. Asked how confident they are that they could quit, the client says 2. What should the social worker do **NEXT**?

A. Ask why the client's importance rating is 9 rather than 5
B. Explain how smoking slows the healing of foot wounds
C. Ask what would raise the confidence rating from 2 to 4

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Asking why a rating is not lower draws out reasons for change and works well when importance is moderate. With importance already at 9, it adds little and leaves the real barrier, low confidence, untouched.

B **Incorrect** (trap: Too directive). Health information can raise importance, but this client already rates importance at 9. Giving facts the client does not need can feel like lecturing and does nothing for the client's low sense of ability.

C **Correct.** Readiness rulers separate importance from confidence, and the lower score shows where to focus (SAMHSA TIP 35; Miller & Rollnick, 2023). A 9 on importance and a 2 on confidence mean the client wants to quit but doubts they can, so the barrier is self-efficacy. Asking what would move confidence up identifies supports and past successes to build on. It beats A because eliciting more reasons strengthens importance, which is already high.

Takeaway: When importance is high and confidence low, assess and build self-efficacy rather than adding reasons to change.

Sources: SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness; Miller & Rollnick (2023). Motivational Interviewing (4th ed.), Importance and confidence rulers
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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82. ASM-1134 (rev 1) · Best answer: D

A hospital social worker meets with a 45-year-old patient who has rheumatoid arthritis. The patient's joint pain is persistent and distressing. For the past year, the patient has spent hours each day reading about joint damage, has gone to the emergency room most weeks fearing disability and has stopped working. Reassurance from the rheumatologist brings relief for only a day. Which diagnosis is **MOST** likely?

A. Illness anxiety disorder
B. Psychological factors affecting other medical conditions
C. Generalized anxiety disorder
D. Somatic symptom disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Illness anxiety disorder centers on fear of having or getting a serious illness when bodily symptoms are absent or mild. This patient has a known disease with persistent pain that is itself the focus of distress.

B **Incorrect** (trap: Near-miss diagnosis). This category applies when psychological or behavioral factors worsen the medical condition, such as skipping treatment. Here the

problem is an excessive response to the symptom, not behavior that worsens the arthritis.

C **Incorrect** (trap: Near-miss diagnosis). Generalized anxiety disorder involves hard-to-control worry about many areas of life, such as work, money and family. This patient's worry centers on one distressing bodily symptom and drives heavy health-related behavior.

D **Correct.** Somatic symptom disorder involves one or more distressing bodily symptoms plus excessive thoughts, feelings or behaviors about them, usually lasting more than 6 months. A real medical illness does not rule it out. The patient's persistent arthritis pain comes with hours of daily research, weekly emergency visits and leaving work. It beats A because illness anxiety disorder applies when bodily symptoms are absent or mild and the fear of illness is the focus, whereas here a prominent symptom drives the distress (DSM-5-TR).

Takeaway: A real medical illness does not exclude somatic symptom disorder; excessive thoughts and behaviors about a distressing symptom define it.

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Somatic Symptom Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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83. ETH-1128 (rev 1) · Best answer: D

A client with PTSD asks the social worker to support a request to the local housing authority for a ground-floor unit. The client signs a release. The housing authority's verification form asks for the client's diagnosis, medications and full treatment history. What is the **MOST** appropriate way to complete the request?

A. Complete every item on the form, since the client signed a release
B. Decline, since housing verification falls outside the treatment role
C. Ask the client's physician to supply the diagnosis instead
D. Verify the disability and the need without listing the diagnosis

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A release makes disclosure lawful, but it does not make every requested detail necessary. The form asks for more than the housing authority needs, and listing the diagnosis and history exposes the client without helping the decision.

B **Incorrect** (trap: Good but not best). Verification letters are a common way to support a client's access to housing. The social worker knows the client's needs and is a reliable third party, so declining leaves the client without help the worker can properly give.

C **Incorrect** (trap: Hands off too soon). The social worker is already well placed to verify the need, so sending the client elsewhere delays the request. A physician would face the same question of how much to disclose.

D **Correct.** In all instances, social workers disclose the least information needed for the purpose (NASW Code 1.07(c)). HUD guidance says housing authorities may request only what is needed to evaluate a disability-related need and must not ask about the nature or severity of a disability (HUD, 2025). A letter confirming that the client has a

disability and needs a ground-floor unit meets the purpose. D beats A because a signed release permits disclosure but does not call for sharing more than the request requires.

Takeaway: A release permits disclosure but does not require sharing more than the purpose needs; verify a need without unnecessary diagnostic detail.

Sources: NASW Code of Ethics (2021), 1.07(c); HUD Housing Choice Voucher Program Guidebook (2025), Fair Housing and Nondiscrimination Requirements: verifying disability-related need
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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84. ETH-1654 (rev 1) · Best answer: A

A 20-year-old comes to a first appointment at a community clinic because the parents insisted. The social worker explains that services are voluntary and the client may decline them. The client folds their arms and says, 'They think I'm depressed. I'm just tired of them running my life.' What should the social worker do **NEXT**?

A. Ask what the client would want to change, if anything
B. End the meeting, since the client is declining services
C. Give a brief depression screen to check the parents' concern

Why each option is right or wrong

A **Correct.** A voluntary adult decides whether to use services, and the social worker helps clients identify and clarify their own goals (NASW Code 1.02). The complaint about being controlled hints at a goal: more say over their own life. Inviting the client to name what they might want, if anything, respects the choice and keeps the door open (Miller & Rollnick, 2023). It beats B because the client has voiced frustration, not a decision to leave; ending now closes off engagement the client may want once their own agenda is heard.

B **Incorrect** (trap: Good but not best). Ending is the right response if, after hearing what is offered, the client chooses to stop. The client has not said that yet. They have described feeling controlled, which is an opening rather than a refusal.

C **Incorrect** (trap: Right action, wrong time). A depression screen may become useful later, with the client's agreement. Starting with the parents' concern repeats the very dynamic the client objects to and is likely to end engagement before it begins.

Takeaway: With a reluctant voluntary client, invite the client's own goals before screening, persuading or closing.

Sources: NASW Code of Ethics (2021), 1.02; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: the client's own goals
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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85. INT-1075 (rev 1) · Best answer: C

A crisis line social worker takes a call from an adult who is crying and talking fast. The worker asks about suicide and violence; the caller has no such thoughts, has no weapons, and is staying at a friend's apartment. The caller has now calmed, says they feel heard, and wants help sorting out 'everything falling apart.' What should the social worker do **NEXT**?

A. Help the caller brainstorm options for the coming week
B. Teach the caller a slow-breathing exercise to settle down
C. Ask what happened today that prompted reaching out
D. Work out a written step-by-step plan for tonight

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Generating alternatives is a later stage of the Roberts model. Without first defining the main problem and what set off the crisis, the options will be scattered and may miss what matters most to the caller.

B **Incorrect** (trap: Good but not best). Calming skills help when a person is too aroused to think, and they fit early in a call. This caller has already settled and feels heard, so stopping for a breathing exercise delays the problem-focused work the caller is asking for.

C **Correct.** Roberts' seven-stage model moves from lethality assessment and rapid rapport to identifying the major problems, starting with the precipitating event: what happened that pushed the person past their usual coping (Roberts & Ottens, 2005). Safety has been checked and the caller feels heard, so the first two stages are done. 'Everything falling apart' is too vague to act on; naming today's trigger focuses the work. It beats A because brainstorming alternatives is stage 5 and needs a defined problem first; options generated now would target a blur.

D **Incorrect** (trap: Right action, wrong time). A concrete action plan is the right step after the problem is defined and options are explored. Building it now skips identifying what triggered the crisis, so the plan may not address the real problem.

Takeaway: In Roberts' model, once safety is assessed and contact is made, define the major problems, starting with the precipitating event, before generating options.

Sources: Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 3: identify major problems, including the precipitating event
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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86. INT-1105 (rev 2) · Best answer: C

During a video session, the social worker glances several times at case records on a second monitor. The client stops mid-sentence and says, 'You keep looking off to the side. I guess I'm pretty boring.' What is the social worker's **BEST** response?

A. Ask what leads the client to see themselves as boring
B. Explain that the notes help track the client's goals
C. Acknowledge reading notes and set them aside

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The comment may reflect a long-standing belief worth exploring, but the client is responding to something that really happened. Asking about the belief before owning the glances implies the client misread the situation. It becomes a useful step after the acknowledgment.

B **Incorrect** (trap: Good but not best). Explaining why the notes were open is partly honest and may reassure the client. It justifies the behavior, though, rather than owning its effect, and it leaves the distraction on screen. The client needs the worker's attention back, not a reason it was elsewhere.

C **Correct.** Congruence means the social worker's words match what the client can see. The client accurately noticed the glances and linked them to feeling boring, so the honest response is to own the behavior, say briefly what it was, and remove the distraction (Rogers, 1957; Hepworth et al.). This confirms the client's observation and restores full attention, which repairs the moment. It beats A because exploring the client's belief about being boring is valuable, but doing it first treats an accurate observation as the client's distortion; it fits once the worker has owned the glances.

Takeaway: When a client accurately notices the worker's distracting behavior, acknowledge it honestly and correct it before exploring the client's interpretation.

Sources: Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, congruence (genuineness) as a core condition; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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87. ETH-1629 (rev 2) · Best answer: B

A social worker at an urban domestic violence program completes intake with a client who identifies as Diné (Navajo) and as Mexican American. The agency database allows more than one race, but staff usually copy the single race on the police report, which lists 'White.' The program director is preparing a report on violence against Indigenous clients. What is the **BEST** way to complete the race entry?

A. Copy the race listed on the police report

B. Enter each race the client names, plus tribal nation

C. Record the Native identity alone so the report counts the client

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Consistency with outside records sounds efficient, but copying a police entry repeats a known source of error. Police and agency records often misclassify Native people, which undercounts the violence they face and weakens the case for services and funding.

B **Correct.** Clients are the experts on their own identity (NASW Code 1.05(c)), and misclassification hides violence against Indigenous people. A national review of cases of missing and murdered Indigenous women found records that listed Native victims as White and police systems that could not search by Native identity (Urban Indian Health Institute, 2018). Recording each identity the client reports, with the tribal nation, keeps the record accurate and lets the director's report count this client. It beats C

because recording the Native identity alone serves the report but overrides how the client identifies; self-identification, recorded in full, serves both accuracy and the count.

C **Incorrect** (trap: Overrides the client's choice). Counting Indigenous clients accurately is the right goal, which makes this tempting. Choosing which identity to record, though, puts the agency's purpose ahead of the client's self-identification and erases the Mexican American identity the client also named; an accurate count comes from recording all of it.

Takeaway: Record race, ethnicity and tribal affiliation as clients self-identify; misclassifying Indigenous people hides the violence they experience.

Sources: Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: racial misclassification; Recommendations; NASW Code of Ethics (2021), 1.05(c)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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88. INT-1571 (rev 2) · Best answer: C

A client with alcohol use disorder attends a weekly outpatient program. Four weeks ago, after drinking rose, the social worker and client revised the plan to add motivational interviewing and a second weekly session. Since then, drinking days have increased from 2 to 5 per week, the client has missed 3 workdays, and the client says, 'I want to stop, but I can't do it on this schedule.' Which action is **BEST**?

A. Explore the client's ambivalence about drinking once more

B. Add a peer recovery coach to the current outpatient plan

C. Refer the client for a higher level of care assessment

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Exploring ambivalence suits a client unsure about change. This client has already done motivational work, wants to stop and is asking for more help, so the barrier is the intensity of care, not motivation.

B **Incorrect** (trap: Good but not best). Peer recovery support is valuable and may belong in the next plan. Adding it at the same level of care, though, is another small adjustment right after the last one failed; with drinking more than doubled and the client asking for more help, the intensity of care needs a formal reassessment.

C **Correct.** Progress monitoring exists to guide decisions: when data show worsening despite a revised plan, care should move toward a more intensive level, guided by the ASAM transition and continued-service criteria (ASAM Criteria, 4th ed.). Drinking has more than doubled, work is affected, and the client asks for more support. Referral fits the Code's direction to refer when a client is not making reasonable progress and other services are needed (NASW Code 1.16(a)). It beats B because another add-on at the same level repeats the approach that just failed while drinking keeps rising.

Takeaway: When monitoring shows worsening despite a revised plan, act on the data by referring for a more intensive level of care.

Sources: American Society of Addiction Medicine (2023). The ASAM Criteria (4th ed.), transition and continued service criteria; NASW Code of Ethics (2021), 1.16(a)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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89. INT-1618 (rev 1) · Best answer: A

A new director at a large homeless shelter plans to move from rule-based discipline to a trauma-informed approach. Staff resist, and many say they will wait to see what one long-time staff member thinks, though that person holds no supervisory title. The board supports the change. What should the director do **FIRST**?

A. Involve that colleague in planning from the start
B. Schedule trauma-informed care training for the full staff
C. Announce a firm start date with the board's backing

Why each option is right or wrong

A **Correct.** Organizations run on an informal structure of influence alongside the formal chart, and informal leaders shape whether staff accept direction (Barnard, 1938). Staff say they are waiting on this colleague, so involving that person in planning builds buy-in before rollout. A trauma-informed approach also calls for collaboration and shared decision-making among staff, not only with clients (SAMHSA, 2014). It beats B because training delivered to a resistant group before influential members are on board is likely to be discounted.

B **Incorrect** (trap: Right action, wrong time). Training is essential to a trauma-informed shift, but it works better once influential staff support the change. Offered first to a resistant group, it may be dismissed.

C **Incorrect** (trap: Too directive). Board backing gives formal authority, but announcing a deadline over staff resistance relies on that authority alone. It can deepen opposition and ignores the informal leadership staff are following.

Takeaway: Organizational change goes better when informal leaders are engaged early; formal authority or training alone meets resistance.

Sources: Barnard, C. I. (1938). The Functions of the Executive. Harvard University Press, Ch. 9: Informal organizations and their relation to formal organizations; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: collaboration and mutuality
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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90. INT-1658 (rev 1) · Best answer: B

A foster parent in a parenting group says praise 'doesn't work' for an 8-year-old who leaves chores unfinished. Asked to describe it, the foster parent says they tell the child 'good job' at bedtime on days when the chores were done. What is the **MOST** appropriate suggestion?

A. Add a larger weekly reward to the bedtime praise
B. Respond right after each chore by naming what was done
C. Take away screen time on any day a chore is left unfinished

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Larger rewards can help, but this one keeps the same flaw: it arrives long after the behavior and does not name it. A reward system works best once praise is immediate and specific.

B **Correct.** Praise works as reinforcement when it comes right after the behavior and names exactly what the child did (Kazdin, 2005). A bedtime 'good job' comes hours late and is vague, so the child may not link it to the chores. Specific, immediate praise, such as thanking the child for putting the dishes away as soon as it happens, fixes the delivery rather than the size of the reward. It beats A because a bigger reward keeps the same delay and vagueness that weakened the praise.

C **Incorrect** (trap: Good but not best). Removing privileges punishes the problem but does not teach or strengthen the wanted behavior, and the praise has not yet had a fair test. Well-delivered reinforcement should come first.

Takeaway: Reinforcement works when it is immediate, specific and contingent; fix how praise is delivered before increasing rewards or adding punishment.

Sources: Kazdin, A. E. (2005). Parent Management Training: Treatment for Oppositional, Aggressive, and Antisocial Behavior in Children and Adolescents. Oxford University Press, positive reinforcement: delivering praise
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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91. INT-1583 (rev 2) · Best answer: C

A family services agency is expanding into a new county and wants to learn which services residents lack. Staff meet one at a time with a pastor, two school principals, a clinic physician and the police chief to hear what problems they see in the community. Which needs assessment method is the agency using?

A. Community forum

B. Focus group

C. Key informant approach

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A community forum is an open public meeting where any resident can speak. It reaches residents directly but can be dominated by the most vocal attendees. Here only selected leaders were consulted.

B **Incorrect** (trap: Wrong model or concept). A focus group brings a small group together for a guided discussion. These leaders were met one at a time, which does not fit a group format.

C **Correct.** The key informant approach collects views from people whose roles give them broad knowledge of a community's problems, such as clergy, school leaders, health providers and police (Rossi, Lipsey & Henry, 2019; Royse, Thyer & Padgett, 2016). It is quick and inexpensive, but it reflects the informants' perspectives, which may differ from residents' own. This beats B because a focus group gathers several people for a guided group discussion, while these leaders were consulted individually.

Takeaway: Key informant needs assessments draw on community leaders' views quickly and cheaply but may miss residents' own perspectives.

Sources: Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 2: Social Problems and Assessing the Need for a Program; Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 3: Needs Assessment
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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92. ASM-1642 (rev 1) · Best answer: B

In an intensive outpatient program, a client describes drinking at a coworker's party after 40 days without alcohol. The client says, 'Everyone was toasting, and refusing would have been rude. Besides, after the week I had, anyone would have had a drink.' Which defense mechanism is the client **MOST** likely using?

A. Denial
B. Rationalization
C. Intellectualization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Denial is refusing to acknowledge a painful reality, such as insisting no drinking happened or that alcohol is not a problem at all. This client openly reports the drink and is explaining it, not denying it.

B **Correct.** Rationalization offers plausible, socially acceptable reasons for a behavior so the person can avoid facing its real cause or meaning. The client admits the drink but explains it as politeness and an understandable reaction to a hard week. It beats A because denial would mean refusing to acknowledge the drinking or its reality, while this client reports it openly and only excuses it (Kaplan & Sadock's Synopsis, 12th ed.).

C **Incorrect** (trap: Wrong model or concept). Intellectualization uses abstract, detached analysis to avoid feelings, such as discussing relapse statistics instead of personal regret. The client gives everyday excuses tied to the situation, not abstract analysis.

Takeaway: Rationalization gives plausible excuses for a behavior the person admits; denial refuses to acknowledge the reality itself.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Theories of personality and psychopathology: defense mechanisms
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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93. INT-1026 (rev 3) · Best answer: A

An employee assistance program refers a 52-year-old client for stress management by video. The client describes long hours and a new manager. The client also mentions chest tightness when climbing stairs at work that eases after resting, and has not seen a physician in 5 years. The client asks to learn relaxation skills. What is the social worker's **BEST** response?

A. Urge the client to get a prompt medical evaluation
B. Teach slow breathing to use when the tightness starts
C. Ask the client to log when the tightness occurs this week

Why each option is right or wrong

A **Correct.** When physical symptoms could signal a medical problem, a medical evaluation comes before stress-management techniques. Chest tightness brought on by exertion and relieved by rest is a typical angina pattern and needs a physician's assessment (American Heart Association), and heart and lung conditions can produce symptoms that look like stress or anxiety (DSM-5-TR). Social workers don't diagnose medical conditions; they route the client to medical care. It beats C because a symptom log sounds careful but spends a week watching a possible heart problem instead of having it checked; a log can support the medical visit, not replace it. Symptoms at rest or worsening call for emergency care.

B **Incorrect** (trap: Ignores risk). Breathing skills may help later, but teaching them for this symptom treats possible chest pain as stress and could delay care for a heart problem. Relaxation training fits once medical causes have been evaluated.

C **Incorrect** (trap: Ignores risk). Tracking symptoms is a reasonable assessment habit, and a log could give a physician useful detail. Here it delays care: chest tightness brought on by exertion needs a physician's evaluation now, and a week of monitoring leaves a possible cardiac problem unchecked.

Takeaway: Before teaching stress-management skills, send possible medical symptoms, such as chest pain on exertion, for medical evaluation; social workers don't diagnose them.

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Anxiety Disorder Due to Another Medical Condition; American Heart Association (2025). Angina (Chest Pain). heart.org, last reviewed Jan 30, 2025, Quick Facts; stable versus unstable angina

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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94. ASM-1093 (rev 2) · Best answer: C

When a caregiver says a young child's injury came from a fall at home, which bruise **MOST** warrants an evaluation for physical abuse?

A. Bruised shins on an active 4-year-old

B. A forehead bruise on a toddler who just began walking

C. Bruising on the ear of a 2-year-old

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bruises over the shins are common in active preschoolers, because the shins hit furniture and the ground during ordinary play. They fit a fall explanation.

B **Incorrect** (trap: Near-miss diagnosis). Newly walking toddlers often fall forward and bruise the forehead, a bony area. This pattern is expected and fits the explanation given.

C **Correct.** Validated bruising rules such as TEN-4-FACESp flag bruises on the torso, ears or neck in children under 4, and any bruise in a young infant, as strongly associated with abuse (Pierce et al., 2021). The ear is a protected site rarely injured in ordinary falls, so a fall explanation does not fit well. It beats B because forehead bruises are expected in toddlers learning to walk; location, not just age, separates typical from concerning bruises.

Takeaway: Bruises on the ears, neck or torso of a child under 4 warrant abuse evaluation; bruises on bony, exposed areas usually reflect accidents.

Sources: Pierce, M. C., et al. (2021). Validation of a clinical decision rule to predict abuse in young children based on bruising characteristics. JAMA Network Open, 4(4), e215832, TEN-4-FACESp bruising regions
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 2 of 3

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95. INT-1665 (rev 1) · Best answer: C

An agency evaluates a 12-week program for adults with alcohol use disorder by comparing intake and final scores. Of 60 clients who started, 25 left before week 12, most of them clients with the most severe drinking at intake. Final scores among those who finished are much better than the group's intake scores. Which threat to internal validity **MOST** likely affects this conclusion?

A. History
B. Maturation
C. Attrition
D. Selection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). History refers to outside events during the program, such as a new law or a community crisis, that could explain change. Nothing here points to an outside event; the problem is who remained to be measured.

B **Incorrect** (trap: Wrong model or concept). Maturation refers to natural change within people over time, such as growth or spontaneous recovery. It may play some role over 12 weeks, but it does not explain why losing the most severe clients makes final scores look better.

C **Correct.** Attrition, sometimes called experimental mortality, is the loss of participants before the final measure; it threatens internal validity when those who leave differ from those who stay (Shadish, Cook & Campbell, 2002). Here 25 of 60 left, mostly those with the most severe drinking, so the final group was healthier from the start, and comparing its scores with the whole group's intake scores builds in apparent improvement. It beats D because selection concerns differences in who enters the groups being compared, while this bias comes from who left one group during the program.

D **Incorrect** (trap: Wrong model or concept). Selection threatens validity when the groups being compared differ from the start, as with nonrandom assignment. This was one group measured twice; the bias comes from losing participants during the program, not from how groups were formed.

Takeaway: When participants who leave a study differ from those who stay, attrition can make a program look more effective than it is.

Sources: Shadish, W. R., Cook, T. D., & Campbell, D. T. (2002). Experimental and Quasi-Experimental Designs for Generalized Causal Inference. Houghton Mifflin, threats to internal validity: attrition
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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96. ETH-0114 (rev 1) · Best answer: A

Which approach to recruiting current clients for an agency research study is **MOST** appropriate?

A. Assure clients that declining has no effect on services
B. Offer participants priority placement on the agency's waitlist
C. Ask each client's own social worker to invite them

Why each option is right or wrong

A **Correct.** Research consent must be voluntary, free of coercion or undue inducement, and clients must know they can decline or withdraw without penalty (NASW Code 5.02(e), 5.02(i)). Current clients may fear that saying no will affect their care, so stating plainly that services do not depend on participation protects voluntariness. Federal human subjects rules likewise require minimizing coercion and undue influence (45 CFR 46.116). It beats C because a client's own social worker holds influence over the client, which makes refusing harder.

B **Incorrect** (trap: Misreads law or ethics). Linking participation to faster access to services is an undue inducement. Clients who need help may feel they have to join to get it, which undermines voluntary consent.

C **Incorrect** (trap: Boundary slip or collusion). A request from the client's own social worker carries the weight of the helping relationship, so clients may agree to please the worker. Having neutral research staff recruit reduces this pressure and the dual-role conflict.

Takeaway: Recruit clients for research so refusal is easy: no link to services and no pressure from the treating social worker.

Sources: NASW Code of Ethics (2021), 5.02(e); NASW Code of Ethics (2021), 5.02(i); 45 CFR Part 46 (Common Rule), 45 CFR 46.116
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** MOST APPROPRIATE · **Cognitive** recall · **Difficulty** 1 of 3

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97. ETH-1684 (rev 1) · Best answer: D

A first-generation college student meets with a campus social worker after failing a math placement exam. The student earned A's in high school math and scores well on practice tests at home. Just before the exam, an advisor remarked that students 'from your neighborhood' usually need remedial math. The student says their mind went blank. Which concept **MOST** likely explains the poor score?

A. Impostor phenomenon
B. Learned helplessness
C. Social anxiety disorder
D. Stereotype threat

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Impostor feelings involve doubting one's own competence despite clear success and fearing exposure as a fraud. Nothing suggests a long-standing sense of fraud, and the failure followed a specific remark about the student's group.

B **Incorrect** (trap: Wrong model or concept). Learned helplessness follows repeated uncontrollable failures that lead a person to stop trying. This student has a record of success and was trying hard; the drop was sudden and tied to the exam setting.

C **Incorrect** (trap: Near-miss diagnosis). Social anxiety disorder centers on fear of scrutiny across social situations, typically lasting 6 months or more (DSM-5-TR). One exam-day episode after a biased remark does not support it.

D **Correct.** Stereotype threat is the risk of confirming a negative stereotype about one's group; when the stereotype is made salient just before a test, that concern can lower performance (Steele & Aronson, 1995). The advisor's remark cast the student's group as weak in math right before the exam, and the student does well when no such cue is present. It beats A because impostor feelings are a lasting sense of being a fraud despite success, not a drop tied to a group stereotype raised at test time.

Takeaway: A negative group stereotype raised before a task can depress performance; recognize stereotype threat instead of attributing the result to ability.

Sources: Steele, C. M., & Aronson, J. (1995). Stereotype threat and the intellectual test performance of African Americans. Journal of Personality and Social Psychology, 69(5), 797-811, whole article; DSM-5-TR (APA, 2022), Anxiety Disorders: Social Anxiety Disorder

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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98. ASM-1513 (rev 2) · Best answer: B

In a fourth session, a client repeatedly accuses the social worker of being cold and judgmental. The client mocks each suggestion and cancels the next appointment at the last minute. As the session goes on, the social worker begins giving short, clipped replies and later tells a supervisor about feeling unusually irritated. Which defense is the client **MOST** likely using?

A. Projection
B. Projective identification
C. Splitting
D. Passive aggression

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Projection attributes one's own unacceptable feelings to another person. The accusations of coldness fit, which makes it the strongest competitor, but projection does not include inducing the other person to feel and act out the projected state.

B **Correct.** Projective identification attributes one's own disowned feelings to another person and then pressures that person, through the interaction, to feel or act them out (Ogden, 1979). The client accuses the social worker of coldness and provokes through mocking and late cancellation, and the worker starts responding coldly, taking on the projected role. It beats A because projection stops at attributing the feeling; the added step here is that the worker begins to experience and enact it. Supervision helps the worker understand and contain the induced feeling.

C **Incorrect** (trap: Wrong model or concept). Splitting sorts people into all-good or all-bad, and the client's picture of the worker as cold and judgmental fits partly. It does

not explain the defining feature here: the worker starts to feel and act out the coldness the client attributes to them.

D **Incorrect** (trap: Wrong model or concept). Passive aggression expresses hostility indirectly, such as through lateness or quiet sabotage. The last-minute cancellation fits partly, but the open accusations and the worker's induced irritation point to a different defense.

Takeaway: Projective identification goes beyond projection: the client induces the other person to feel and act out the disowned feeling.

Sources: Ogden, T. H. (1979). On projective identification. International Journal of Psycho-Analysis, 60, 357-373, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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99. ETH-0110 (rev 1) · Best answer: D

After an assessment, a social worker diagnoses a client with adjustment disorder. The client's insurance plan pays for only a few sessions for that diagnosis, so the client asks the social worker to record major depressive disorder instead. The client does not meet criteria for major depressive disorder. What is the **BEST** response?

A. Record the requested diagnosis to keep the client in care
B. Refer the client to a provider who accepts self-pay clients
C. Explain that billing a false diagnosis is insurance fraud
D. Keep the accurate diagnosis and discuss payment options

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Recording a diagnosis the client does not meet misrepresents the client's condition to the insurer. Good intentions do not change that this is fraud, and the false diagnosis also follows the client in future records.

B **Incorrect** (trap: Hands off too soon). Referral hands the client off over a payment problem the social worker can help solve. Referral may fit if no affordable option exists here, but it should follow a discussion of fees and alternatives.

C **Incorrect** (trap: Too directive). The statement is true, but leading with fraud warnings lectures a client who made an understandable request. It leaves the real problem, affording continued care, unaddressed and can damage the relationship.

D **Correct.** Billing and diagnosis must accurately reflect the services and the client's condition; recording a diagnosis the client does not meet is fraud, even when meant to help (NASW Code 3.05, 4.04). Keeping the accurate diagnosis while exploring options, such as a sliding fee, an appeal or lower-cost services, protects honesty and addresses the client's real concern, paying for care (NASW Code 1.13(a)). It beats C because naming the fraud is accurate but stops at a warning; D declines the request and still helps the client keep getting services.

Takeaway: Diagnose and bill only what the assessment supports; address cost problems through fees, appeals or resources, never by changing the diagnosis.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 1.13(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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100. INT-1515 (rev 2) · Best answer: B

Before a parent's hospital discharge, an adult child asks the social worker, 'Should I move my parent in with me or choose a nursing facility? Just tell me what to do.' The parent has moderate neurocognitive disorder and has said they want to stay with family. The adult child works full time and has two teenagers at home. What is the social worker's **BEST** response?

A. Recommend the nursing facility given the caregiving demands described
B. Help the family weigh both options, including the patient's wishes
C. Schedule a family meeting so the care team can recommend a plan

Why each option is right or wrong

A **Incorrect** (trap: Too directive). A recommendation answers the request but takes over a values-laden family decision and sets aside the parent's stated wishes. The worker can share information about each option without choosing for the family.

B **Correct.** In problem solving, the social worker's role is to strengthen the client's own capacity to decide, not to decide for them (Perlman, 1957; NASW Code 1.02). Walking through each option, including the parent's wish to stay with family, caregiving hours, the teenagers' needs, and costs, helps the adult child reach a choice they can carry out. It beats C because a family meeting can add facts, but framing it as the team recommending a plan hands the decision to professionals instead of building the family's own judgment.

C **Incorrect** (trap: Overrides the client's choice). A family meeting with the team is useful for medical facts and options. Setting it up so the team recommends the plan shifts the decision away from the family, who must live with it.

Takeaway: In problem solving, the social worker helps clients weigh options against their own values and limits rather than deciding for them.

Sources: Perlman, H. H. (1957). Social Casework: A Problem-Solving Process. University of Chicago Press, the problem-solving process; NASW Code of Ethics (2021), 1.02
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.4 What the social worker does in problem solving · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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101. ETH-1694 (rev 1) · Best answer: C

A school social worker notices that a 9-year-old who recently arrived from Nigeria has started asking classmates to call them 'Sam' instead of their Yoruba name. Since the first week, the teacher has shortened the name, saying it is 'too hard to say.' The child seems quieter in class. What should the social worker do **FIRST**?

A. Ask the teacher to use the child's full Yoruba name
B. Lead a class lesson on names and their meanings
C. Ask the child privately about the name they want and why
D. Encourage the child to correct the teacher's pronunciation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Talking with the teacher is likely needed, and soon. Asking for the full name before hearing from the child risks overriding the child's own preference, which may have been chosen freely.

B **Incorrect** (trap: Good but not best). A lesson that values names can improve the class climate later. Done first, it may spotlight a child who is already withdrawing, without knowing what the child wants.

C **Correct.** Repeatedly shortening a student's name because it seems hard to say is a racial microaggression that can teach children their identity is a burden (Kohli & Solórzano, 2012). Still, the new nickname could be a free choice or a response to that message, and only the child can say which. Asking privately treats the child as the expert on their own identity (NASW Code 1.05(c)). It beats A because pressing the teacher to use the full name could override the child's choice; that step should follow what the child says.

D **Incorrect** (trap: Too directive). Asking a quiet 9-year-old to correct an adult puts the burden of the teacher's behavior on the child and may add stress. Self-advocacy coaching can come later, with support, if the child wants it.

Takeaway: When a microaggression may have shaped a child's choices, ask the child what they want before acting on their behalf.

Sources: Kohli, R., & Solórzano, D. G. (2012). Teachers, please learn our names!: Racial microaggressions and the K-12 classroom. Race Ethnicity and Education, 15(4), 441-462, whole article; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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102. ASM-1164 (rev 2) · Best answer: A

A client stopped taking paroxetine 2 days ago after losing insurance coverage. The client now has dizziness, nausea, irritability, and brief 'electric zaps' in the head, and fears the depression is coming back. What is the social worker's **BEST** response?

A. Help the client reach the prescriber today about the stop and cost
B. Explain that these symptoms are harmless and will fade soon
C. Arrange an emergency evaluation for possible serotonin syndrome

Why each option is right or wrong

A **Correct.** Stopping an antidepressant abruptly, especially one that leaves the body quickly such as paroxetine, can cause a discontinuation syndrome with dizziness, nausea, irritability and electric-shock sensations (DSM-5-TR). It is distressing but rarely dangerous, and the prescriber can restart the drug, plan a taper or choose a lower-cost option. Linking the client today addresses both the symptoms and the insurance barrier that caused them. It beats C because serotonin syndrome follows adding or raising a serotonergic drug, not stopping one, so emergency care is not needed here. It beats B because reassurance leaves the medication decision and relapse risk unaddressed.

B **Incorrect** (trap: Good but not best). Discontinuation symptoms often ease within a few weeks, so this is partly accurate. But the client also risks depressive relapse off medication, and judging medical symptoms harmless is not the social worker's call; the prescriber should weigh in.

C **Incorrect** (trap: Skips assessment). Serotonin syndrome follows adding or increasing serotonergic drugs, not stopping one, and it brings fever, agitation and muscle jerks. Emergency care is right for those signs; here a same-day prescriber contact fits.

Takeaway: Symptoms after abruptly stopping an antidepressant call for prompt prescriber contact; address the barrier, such as cost, that caused the stop.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Antidepressant Discontinuation Syndrome
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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103. ASM-1661 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

During a hospital family meeting, the social worker asks how the patient manages the stairs at home. A trained interpreter and the patient then talk back and forth in the patient's language for about a minute. The interpreter turns to the social worker and says, 'Basically, they're fine with the stairs.' What is the **MOST** appropriate response?

A. Ask the interpreter to interpret everything both people said
B. Accept the summary and move on to the next question
C. Request a different interpreter for the rest of the meeting

Why each option is right or wrong

A **Correct.** Interpreters render each speaker's words completely so that every party knows exactly what was said; national standards for health care interpreters make accuracy central (NCIHC, 2005). A minute-long exchange reduced to 'basically fine' may hide hesitation, conditions or details about falls. Politely asking for a full interpretation of the exchange, and reminding the interpreter to interpret everything going forward, keeps the patient's own words in the assessment. It beats C because a trained interpreter who summarized once can usually correct course after a brief reminder; replacing them is premature.

B **Incorrect** (trap: Misuses the technique). Accepting a summary lets the interpreter filter the patient's answer. The minute of discussion may include worries about stairs

that affect discharge safety. Clients are entitled to full communication in their language (NASW Cultural Competence Standard 9).

C **Incorrect** (trap: Hands off too soon). Replacing the interpreter may be needed if summarizing continues after a clear request. As a first response it is premature and would disrupt the meeting when a brief reminder is likely to work.

Takeaway: Interpreters should render everything said; when an interpreter summarizes, ask for a full interpretation of the exchange.

Sources: National Council on Interpreting in Health Care (2005). National Standards of Practice for Interpreters in Health Care, Accuracy standards; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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104. ASM-1042 (rev 1) · Best answer: C

Parents bring their 16-year-old to an outpatient family clinic after 4 months of falling grades and pulling away from friends. The teen says people at school seem to send hidden signals and sometimes a faint voice calls their name, but adds that it might be their mind playing tricks. The parents believe it is a phase. What is the social worker's **BEST** recommendation?

A. Monitor the symptoms and reassess at monthly visits
B. Begin family sessions to support the teen at school
C. Refer the teen for a specialized early psychosis evaluation

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Monitoring is reasonable for vague, isolated changes. Here, perceptual changes and ideas of hidden signals with declining function warrant specialist evaluation now; waiting risks a longer period of untreated psychosis if symptoms progress.

B **Incorrect** (trap: Good but not best). Family support helps, and families are part of early psychosis care. Family sessions alone would not evaluate the emerging psychotic symptoms that drive the concern.

C **Correct.** Mild psychotic-like experiences, such as ideas that others send hidden signals and faint voices while doubt is still intact, combined with falling function in adolescence, are warning signs of possible emerging psychosis (DSM-5-TR, attenuated psychosis syndrome). Coordinated specialty care for early psychosis improves outcomes, especially when untreated psychosis is brief (Kane et al., 2016). It beats A because waiting month to month delays evaluation during the period when early intervention helps most.

Takeaway: Teens with mild psychotic-like experiences and falling function need prompt specialized early psychosis evaluation, not watchful waiting.

Sources: DSM-5-TR (APA, 2022), Conditions for Further Study: Attenuated Psychosis Syndrome; Kane, J. M., et al. (2016). Comprehensive versus usual community care for first-episode psychosis: 2-year outcomes from the NIMH RAISE Early Treatment Program. American Journal of Psychiatry, 173(4), 362-372, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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105. ETH-1193 (rev 2) · Best answer: A

After a flood, a county mental health agency asks staff to add evening shifts at evacuation shelters. A social worker whose own home flooded is staying with relatives, sleeping poorly, and handling insurance claims. The social worker wants to help and has not told the supervisor about the damage. What is the **BEST** action for the social worker?

A. Tell the supervisor about the losses and agree on limited shifts
B. Work the shifts and seek supervision if stress becomes a problem
C. Ask the supervisor to excuse the social worker from shelter work

Why each option is right or wrong

A **Correct.** The NASW Code calls professional self-care paramount for competent, ethical practice (Purpose) and asks social workers to care for themselves professionally and personally (Integrity). It also asks them to serve in public emergencies to the greatest extent possible (6.03), which allows for personal limits. Telling the supervisor about the damage lets the agency plan shifts the social worker can sustain. It beats B because waiting to seek supervision until stress shows lets known risks, poor sleep and personal losses, reach distressed residents first.

B **Incorrect** (trap: Ignores risk). Planning to seek supervision later sounds responsible, but it waits for problems to show while the social worker is already sleeping poorly and handling personal losses. Telling the supervisor now lets the agency set sustainable shifts before strain affects distressed residents.

C **Incorrect** (trap: Good but not best). Stepping away entirely may be needed if the social worker cannot function, but nothing in the stem suggests that. Asking to be excused from all shelter work gives up help the social worker wants to offer and could safely provide in part.

Takeaway: In emergencies, serve within sustainable limits: tell your supervisor about your own losses and plan a workload you can safely carry.

Sources: NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 6.03
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.7 Principles of self-care for the social worker · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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106. ASM-1518 (rev 1) · Best answer: A

One view holds that barriers in buildings, policies and attitudes limit people more than their impairments do. Which model of disability is this?

A. Social model
B. Medical model
C. Moral model

Why each option is right or wrong

A **Correct.** The social model of disability distinguishes impairment, a feature of the body or mind, from disability, the exclusion created by barriers such as inaccessible buildings, rigid policies and negative attitudes (Oliver, 1990). Because the problem sits in society, the response targets those barriers. It beats B because the medical model locates disability in the individual's impairment and focuses on diagnosis, treatment or cure. Social workers draw on the social model when advocating for accommodations and accessible services.

B **Incorrect** (trap: Wrong model or concept). The medical model treats disability as a problem within the person, to be diagnosed, treated or cured by professionals. The question places the source of disability in the environment, which is the reverse emphasis.

C **Incorrect** (trap: Wrong model or concept). The moral model, the oldest view, treats disability as a sign of sin, a test of faith or a moral failing. It locates meaning in the person's character, not in environmental barriers.

Takeaway: The social model locates disability in environmental and attitudinal barriers; the medical model locates it in the person's impairment.

Sources: Oliver, M. (1990). The Politics of Disablement. Macmillan, social model of disability
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.17 Effects of physical and mental ability across the lifespan · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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107. ETH-1593 (rev 1) · Best answer: C

A hospice social worker meets with the adult children of a Black patient with advanced cancer. The family declines a hospice admission, saying doctors 'gave up on' a relative decades ago because of race. A team member describes the family as 'in denial.' What is the **MOST** helpful response by the social worker?

A. Review hospice benefits and outcome data with the family
B. Document the family's denial and revisit hospice next week
C. Acknowledge the past harm and ask what would build trust

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Clear information matters and can follow once trust grows. Leading with data treats the decision as a knowledge gap and passes over the experience of racism the family just described.

B **Incorrect** (trap: Wrong model or concept). Labeling the family 'in denial' pathologizes a reasonable response to past discrimination. It also delays the conversation the family needs now about their experience and goals.

C **Correct.** Anti-racist practice recognizes that mistrust of health care among Black families often rests on real experiences of racism, not denial, and treats families as experts on their own experience (NASW Code 1.05(c)). The family names a specific past harm. Acknowledging it and asking what would help them trust the team respects that experience and keeps decisions with the family. It beats A because offering data before addressing the mistrust answers a question the family did not ask and can feel like pressure.

Takeaway: Treat mistrust rooted in experiences of racism as legitimate; acknowledge it before offering information or recommendations.

Sources: NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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108. INT-0006 (rev 1) · Best answer: B

In the third meeting of a closed anger management group, several members complain that the attendance rules are unfair and question the leader's competence. The social worker calmly restates the group agreements, but the complaints continue. What should the social worker do **NEXT**?

A. Restate the rules firmly and move on to the planned skill
B. Invite the group to discuss its reactions to the rules
C. Meet outside the group with the members who complain

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Repeating a step that has already failed escalates a power struggle. Members may hear it as the leader avoiding their concerns, which deepens the challenge to the leader.

B **Correct.** Members challenging rules and testing the leader in early meetings is typical of the storming, or power and control, stage (Tuckman, 1965; Garland, Jones & Kolodny, 1965). Restating the rules has not settled it, so the next step is to open the conflict for discussion without defensiveness. Working through it builds trust and norms (Yalom & Leszcz, 2020). It beats C because private meetings move a group-level process out of the group, which can create subgroups and keep members from learning to handle conflict together.

C **Incorrect** (trap: Good but not best). Individual meetings may help with a member who keeps disrupting after the group has addressed it. As a first response, they pull a normal group process outside the group and can create subgroups.

Takeaway: Early challenges to rules and the leader signal the storming stage; address the conflict openly in the group without defensiveness.

Sources: Tuckman, B. W. (1965). Developmental sequence in small groups. Psychological Bulletin, 63(6), 384-399, storming stage; Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, early group development: conflict; Garland, J. A., Jones, H. E., & Kolodny, R. L. (1965). A model for stages of development in social work groups. In S. Bernstein (Ed.), Explorations in Group Work. Boston University School of Social Work, power and control stage
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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109. ETH-1529 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

An adult with an intellectual disability chooses trusted people who help them understand options and communicate choices, while the adult keeps the final say. Which arrangement does this describe?

A. Supported decision-making
B. Limited guardianship
C. Substituted judgment
D. Health care power of attorney

Why each option is right or wrong

A **Correct.** Supported decision-making lets a person with a disability choose supporters who help them understand information and express choices, while the person keeps legal authority to decide (United Nations Convention on the Rights of Persons with Disabilities, Article 12). It is a less restrictive alternative to guardianship and fits the duty to enhance a client's own ability to decide (NASW Code 1.03(c)). It beats B because even a limited guardianship moves some decisions to a court-appointed person; here the adult keeps the final say.

B **Incorrect** (trap: Wrong model or concept). A limited guardianship is court ordered and moves specific decisions to a guardian. It is narrower than full guardianship, but in this arrangement nobody else holds decision authority, so it is not guardianship of any kind.

C **Incorrect** (trap: Wrong model or concept). Substituted judgment is a standard a surrogate uses when the person cannot decide: the surrogate chooses what the person would have chosen. Here the adult makes the decision with help, so no surrogate is involved.

D **Incorrect** (trap: Wrong model or concept). A health care power of attorney names an agent to make medical decisions if the person later loses capacity. It covers health care and transfers authority when it applies; here the adult keeps authority over all choices.

Takeaway: Supported decision-making keeps authority with the person while chosen supporters help; it is a less restrictive alternative to guardianship.

Sources: United Nations Convention on the Rights of Persons with Disabilities (2006), Article 12: Equal recognition before the law; NASW Code of Ethics (2021), 1.03(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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110. ASM-1637 (rev 1) · Best answer: C

Parents of a 6-year-old in treatment for leukemia ask the hospital social worker for help with their 10-year-old, who has started skipping homework and slamming doors. Since the diagnosis, the parents have spent most nights at the hospital, and relatives have cared for the 10-year-old. The parents want a reward chart for school behavior. What should the social worker do **FIRST**?

A. Help the parents set up a reward chart for finishing homework
B. Refer the 10-year-old for an evaluation for ADHD
C. Hear from the 10-year-old how home life has changed

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A reward chart may help later, once the reasons for the behavior are understood. Starting with it treats the homework and door slamming as the problem and skips the family change that likely explains them.

B **Incorrect** (trap: Skips assessment). An ADHD evaluation would be considered if attention problems were longstanding and present across settings. These changes began after the sibling's diagnosis and the family's upheaval, so a new disorder is not the first explanation to pursue.

C **Correct.** A child's serious illness reorganizes the whole family. Parents' attention and routines shift to treatment, and well siblings often report reduced parental attention, distress and school problems in the first years after a diagnosis (Alderfer et al., 2010). The 10-year-old's behavior began after the diagnosis and the parents' absence, so the first step is to hear from the sibling about what has changed. It beats A because a reward chart targets the behavior without knowing what drives it and may miss the sibling's distress and need for connection.

Takeaway: When one child is seriously ill, assess how the family's reorganization affects well siblings before targeting their behavior.

Sources: Alderfer, M. A., Long, K. A., Lown, E. A., Marsland, A. L., Ostrowski, N. L., Hock, J. M., & Ewing, L. J. (2010). Psychosocial adjustment of siblings of children with cancer: A systematic review. Psycho-Oncology, 19(8), 789-805, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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111. ETH-1126 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A community mental health center's director wants to feature a current client's recovery story, with a photo and first name, in the agency's annual fundraising report. In a session, the client says they would be proud to take part and that telling their story supports their recovery. What is the **BEST** way for the social worker to proceed?

A. Decline, since featuring current clients in publicity is improper

B. Include the story and note the client's agreement in the chart

C. Obtain written authorization after explaining that saying no is fine

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Featuring a current client does carry a risk of pressure, so caution is right. But a competent adult may choose to share their own story. Refusing outright decides for the client instead of making sure the choice is free and informed.

B **Incorrect** (trap: Misreads law or ethics). A verbal yes noted in the chart is not a valid authorization to publish a photo and name. A written authorization should state what will be shared, where and for how long, and the client may revoke it.

C **Correct.** Client information is disclosed only with valid consent (NASW Code 1.07(b)), and HIPAA requires a written authorization for a use like publicity (45 CFR 164.508(a)(1)). Because the request comes from the agency serving the client, the social worker makes sure the client knows that declining will not affect services, so the

agency does not take unfair advantage of the relationship (1.06(b)). C beats A because the client is a competent adult who wants to share the story; a blanket refusal overrides that choice when a free, informed authorization can protect the client.

Takeaway: A client may share their story publicly, but only through a free, informed, written authorization naming what will be used.

Sources: NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.06(b); HIPAA Privacy Rule, 45 CFR 164.508(a)(1)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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112. INT-1094 (rev 2) · Best answer: A

An employee assistance client drinks 4 to 5 beers most weeknights and wants to cut back to 2 drinks on weekend nights only. The client says quitting entirely is not the goal right now but is willing to keep a daily log. What is the **MOST** appropriate step?

A. Set the drinking limit with the client and review it weekly

B. Have the client log drinks for 2 weeks before setting a limit

C. Support the goal if the client agrees to attend AA meetings

Why each option is right or wrong

A **Correct.** Harm reduction treats any positive change as progress and lets the client set the goal, with abstinence as one option rather than a requirement (Marlatt & Witkiewitz, 2002). This client wants a specific, sizable reduction and offers to track it. Building a concrete plan with a weekly review supports autonomy and incremental change (Hawk et al., 2017). It beats B because a 2-week baseline before any limit puts off a change the client is ready to make now; the daily log works best alongside the agreed limit, as the record reviewed each week. If moderation does not work, the plan can be revisited together.

B **Incorrect** (trap: Right action, wrong time). Self-monitoring is useful, and the client already offers to keep a log. Holding off on the goal for 2 weeks of baseline, though, delays a specific change the client is ready to start; the log can begin now and track progress toward the agreed limit.

C **Incorrect** (trap: Overrides the client's choice). Mutual-help groups can be useful, but making them a condition of support overrides the client's choice. Harm reduction offers options without coercion; AA can be suggested, not required.

Takeaway: Harm reduction accepts client-defined goals such as moderation, supports measurable steps, and treats any reduction in harm as progress.

Sources: Marlatt, G. A., & Witkiewitz, K. (2002). Harm reduction approaches to alcohol use: Health promotion, prevention, and treatment. Addictive Behaviors, 27(6), 867-886, whole article; Hawk, M., et al. (2017). Harm reduction principles for healthcare settings. Harm Reduction Journal, 14, 70, Table 1: autonomy; incrementalism

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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113. ETH-1615 (rev 2) · Best answer: D

Which term says that race, gender, class and other identities combine to shape unique experiences of privilege and oppression?

A. Double jeopardy
B. Cultural pluralism
C. Minority stress
D. Intersectionality

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Double jeopardy describes facing two forms of discrimination at once, treated as additive. Intersectionality goes further, holding that combined identities create distinct experiences that adding single categories cannot capture.

B **Incorrect** (trap: Wrong model or concept). Cultural pluralism describes a society in which distinct cultural groups keep their identities while living together. It concerns how groups coexist, not how one person's identities interact.

C **Incorrect** (trap: Wrong model or concept). Minority stress refers to chronic strain from stigma tied to a minority identity, such as expecting rejection. It addresses the effects of a stigmatized identity, not how several identities combine.

D **Correct.** Intersectionality, a term introduced by legal scholar Kimberlé Crenshaw, holds that identities such as race, gender and class interact, so a Black woman's experience cannot be understood by adding race and gender separately (Crenshaw, 1989). The Code asks social workers to understand oppression across these dimensions (NASW Code 1.05(d)). It beats A because double jeopardy describes stacked, additive disadvantage, which is the single-axis sum that intersectionality says misses how identities shape each other.

Takeaway: Intersectionality: interacting identities create distinct experiences of privilege and oppression; assess them together, not one category at a time.

Sources: Crenshaw, K. (1989). Demarginalizing the intersection of race and sex: A Black feminist critique of antidiscrimination doctrine, feminist theory and antiracist politics. University of Chicago Legal Forum, 1989(1), 139-167, whole article; NASW Code of Ethics (2021), 1.05(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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114. ASM-1180 (rev 1) · Best answer: A

A 15-year-old in foster care was sexually abused for years by a stepparent while the client's other parent knew and did nothing. In sessions, the client says adults 'only pretend to care,' breaks rules to test each new foster parent, and refuses to lean on any staff member. In Finkelhor and Browne's model, which traumagenic dynamic do these reactions **MOST** likely reflect?

A. Betrayal
B. Powerlessness
C. Stigmatization
D. Traumatic sexualization

Why each option is right or wrong

A **Correct.** Finkelhor and Browne (1985) describe four traumagenic dynamics of child sexual abuse. Betrayal arises when someone the child depended on causes harm or fails to protect, and it shows up as distrust, testing and refusing to depend on caregivers. A parent who knew and did nothing, followed by testing foster parents and rejecting staff, fits betrayal. It beats B because powerlessness centers on fear, helplessness or a need for control rather than on distrust of caregivers.

B **Incorrect** (trap: Wrong model or concept). Powerlessness comes from repeated invasion of the body against the child's will. It tends to produce anxiety, fear, a sense of helplessness or efforts to control. Rule testing can look like control, but the client's words point to distrust of caregivers.

C **Incorrect** (trap: Wrong model or concept). Stigmatization refers to shame, guilt and a sense of being damaged or different, often leading to isolation or self-harm. The client's statements are about whether adults can be relied on, not about personal shame.

D **Incorrect** (trap: Wrong model or concept). Traumatic sexualization involves developmentally inappropriate sexual knowledge, behavior or confusion about sex. Nothing in the stem describes sexual behavior or confusion.

Takeaway: Abuse by or with the knowledge of a trusted caregiver can produce betrayal reactions: distrust, testing and refusal to depend on adults.

Sources: Finkelhor, D., & Browne, A. (1985). The traumatic impact of child sexual abuse: A conceptualization. American Journal of Orthopsychiatry, 55(4), 530-541, traumagenic dynamics
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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115. ETH-1022 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A social worker trained in prolonged exposure therapy has provided supportive therapy for 4 months to an adult client with posttraumatic stress disorder. The social worker now believes prolonged exposure would help and wants to begin it. What should the social worker do **FIRST**?

A. Discuss the approach, its risks and alternatives with the client
B. Have the client sign an updated treatment consent form
C. Begin with a short trial exposure to gauge the response

Why each option is right or wrong

A **Correct.** Informed consent is an ongoing process that is revisited whenever services change. Before starting a new treatment, the social worker explains its purpose, likely benefits and risks, such as a temporary rise in distress, reasonable alternatives and the client's right to refuse, and gives the client a chance to ask questions (NASW Code 1.03(a)). A beats B because a signature without discussion records agreement without making sure the client understands what exposure involves.

B **Incorrect** (trap: Misuses the technique). An updated signature may be part of the process, but signing alone does not ensure understanding. The discussion of purpose, risks and alternatives comes first; the form documents it.

C **Incorrect** (trap: Right action, wrong time). A trial exposure is still the new treatment. Starting it before the client understands and agrees to the approach bypasses informed consent, even if the session is short.

Takeaway: Revisit informed consent whenever treatment changes: explain the new approach, its risks and alternatives before starting.

Sources: NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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116. ETH-0105 (rev 2) · Best answer: C

A program director asks a social worker to start offering eye movement desensitization and reprocessing (EMDR) to clients next month. The social worker already treats several clients with trauma using cognitive behavioral methods but has had no EMDR training. A trained EMDR provider works at a clinic across town. What is the **MOST** appropriate response?

A. Begin EMDR using an online training video as a guide
B. Transfer current trauma clients to the trained EMDR provider
C. Explain the need for formal training and supervision first

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). A video can introduce a method, but it does not replace formal training and supervised practice. Using EMDR on that basis puts clients at risk and practices outside the social worker's competence.

B **Incorrect** (trap: Hands off too soon). Transferring ongoing clients disrupts relationships with people the social worker is already competent to treat with cognitive behavioral methods, and it does not answer the director's request. Referral to the trained provider fits a specific client who needs EMDR before the social worker completes training.

C **Correct.** Social workers offer new techniques only after appropriate study, training, consultation and supervision from people competent in them (NASW Code 1.04(b)). Accepting the assignment is fine if the social worker commits to gaining that competence before using EMDR with clients (NASW Code 4.01(a)). Explaining this to the director keeps clients safe and gives the agency a workable plan. It beats B because transferring current trauma clients abandons people the social worker already serves competently; referral fits only a client who needs EMDR now.

Takeaway: Use a new technique only after formal training and supervision; until then, serve clients within current competence or refer when needed.

Sources: NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 4.01(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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117. INT-1507 (rev 1) · Best answer: C

A 6-year-old placed in an emergency foster home meets a child welfare social worker for the first time. The child hides behind a chair and will not answer questions. Crayons and toy animals sit on a small table in the room. What should the social worker do **FIRST** to build rapport?

A. Ask the child gentle questions about life at home
B. Explain in simple words why the child is in this home
C. Sit nearby and begin playing quietly with the toys

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Questions about home are part of the work, but asked of a hiding child they add pressure and may feel like a test. Questions about possible maltreatment also belong to a trained forensic interviewer, not a rapport visit.

B **Incorrect** (trap: Right action, wrong time). A simple explanation of the placement matters and should come soon. Given to a child who is hiding and silent, it is unlikely to register; engagement through play makes it possible.

C **Correct.** Young children build trust and communicate through play more readily than through talk. Sitting at the child's level and playing quietly lets the child approach at their own pace, which lowers fear in a frightened, newly placed 6-year-old (Webb, 2019). It beats B because an explanation, even a simple one, asks a frightened child to listen before any trust exists; it lands better once the child has engaged.

Takeaway: With young children, build trust through play at the child's own pace before asking questions or giving explanations.

Sources: Webb, N. B. (2019). Social Work Practice with Children (4th ed.). Guilford Press, engaging children; play as communication

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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118. ETH-1500 (rev 1) · Best answer: B

During a home visit, an 84-year-old client who lives alone on a fixed income hands the social worker a holiday card with $150 in cash inside. The client says the social worker has been 'like family' this year and would be hurt by a refusal. What is the social worker's **BEST** response?

A. Accept the cash and give it to the agency's client fund
B. Decline the cash warmly and talk about what the gift means
C. Accept the cash to spare the client's feelings
D. Return the cash later by mail with a policy note

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Passing the money to an agency fund feels like a compromise, but the client still loses $150 they may need, and the meaning of the gesture goes unexplored. Some agencies accept gifts made to the organization itself; this gift was personal.

B **Correct.** The Code has no gift standard, so gifts are weighed as conflict-of-interest and boundary questions: value, timing, motive and the client's means (NASW Code 1.06(a)).

Cash worth $150 from a client on a fixed income is a real financial benefit, so the social worker declines it. Declining in person, with warmth, and talking about what the gesture means honors the client's feelings and protects the relationship. It beats A because handing the money to the agency still takes it from the client and leaves its meaning unaddressed (Reamer, 2020).

C **Incorrect** (trap: Boundary slip or collusion). Accepting a meaningful sum to avoid hurt feelings puts the worker's comfort ahead of the client's interests and blurs the professional role. A small token, such as the card itself, could be kept; the cash is different.

D **Incorrect** (trap: Misuses the technique). Returning the money is the right decision carried out the wrong way. A mailed return with a policy note can feel like rejection and skips the conversation in which the client's gratitude and isolation can be acknowledged.

Takeaway: Weigh gifts by value, motive and the client's means; decline money or valuable gifts warmly and discuss their meaning.

Sources: NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts from clients
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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119. INT-1160 (rev 1) · Best answer: C

A client asks for family therapy because of escalating conflict with their partner, who refuses to attend any sessions. The social worker practices from a Bowen family systems approach. The client wants to start now and says, 'I'm the one who keeps losing it.' What is the **BEST** way to proceed?

A. Wait to begin until the partner agrees to come to sessions
B. Refer the client for individual psychodynamic treatment
C. Coach the client alone to lower their own reactivity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Some family approaches need the whole family present. Bowen work does not, and making the reluctant partner's attendance a condition leaves the motivated client without help.

B **Incorrect** (trap: Hands off too soon). An individual referral abandons the systems lens the client asked for and the social worker can provide. Bowen coaching with one member is a recognized family therapy format, so no hand-off is needed.

C **Correct.** Bowen theory holds that a change in one motivated member's differentiation can shift the whole system, so Bowenian therapists often work with one person (Bowen, 1978; Carter & Orfanidis, 1976). The client is ready, sees their own part, and the partner will not attend. Coaching the client to observe the pattern, manage emotional reactivity, and take calm I-positions is family systems work even with one person in the room. It beats A because waiting for the reluctant partner delays help the client wants and is not required by the model.

Takeaway: Bowen therapy can work through one motivated family member; raising that person's differentiation can change the whole system's pattern.

Sources: Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, differentiation of self; therapy with one family member; Carter, E., & Orfanidis, M. M. (1976). Family therapy with one person and the family therapist's own family. In P. J. Guerin (Ed.), Family Therapy: Theory and Practice. Gardner Press, whole chapter
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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120. ETH-1706 (rev 1) · Best answer: B

In a children's hospital, Deaf parents who use American Sign Language learn that their 10-month-old has profound hearing loss. After a full discussion with the audiologist, they decline cochlear implant surgery. They plan to raise the child in ASL within the Deaf community and have joined a family sign program. The surgeon asks the social worker to 'help them see what is best' and mentions neglect. What is the **MOST** appropriate response?

A. Encourage the parents to reconsider, citing spoken-language outcomes
B. Support the parents' decision and their plan for early ASL
C. Request an ethics consultation to resolve the disagreement
D. File a medical neglect report with the child welfare agency

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Parents deserve accurate information about implant outcomes, and the audiologist has already provided it. Pressing them to change an informed choice treats the hearing world's norm as the only good outcome and dismisses the family's culture.

B **Correct.** Many Deaf people experience deafness as a cultural and linguistic identity, and ASL is a full language that deaf children acquire on a typical timeline when exposed early (Humphries et al., 2012). These parents made an informed choice and have already arranged early ASL exposure, which protects the child from language deprivation. Respecting culture as a strength and the family's informed decision follows NASW Code 1.05(a) and 1.02. It beats C because an ethics consultation fits a real impasse over a child's welfare; here the plan gives the child full early language access, so no override is in question.

C **Incorrect** (trap: Hands off too soon). Ethics consultation helps when a team and family face a true impasse over a child's welfare. The parents' plan provides full early language access through ASL, so no override is in question, and referring the matter signals that the family's choice is suspect.

D **Incorrect** (trap: Misreads law or ethics). Medical neglect involves failing to provide care needed to prevent serious harm. Cochlear implants are elective, and a plan for early, full exposure to ASL addresses language development, so a report would not fit the usual threshold and would damage trust.

Takeaway: Deafness can be a cultural and linguistic identity; support informed parental choices that secure early, full language access rather than pressing medical norms.

Sources: Humphries, T., Kushalnagar, P., Mathur, G., Napoli, D. J., Padden, C., Rathmann, C., & Smith, S. R. (2012). Language acquisition for deaf children: Reducing the harms of zero tolerance to the use of alternative approaches. Harm Reduction Journal, 9, 16, whole article; NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.02

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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121. ASM-1167 (rev 1) · Best answer: A

A 79-year-old client has cared for a spouse with advanced Parkinson's disease for 6 years. In a caregiver support session, the client says, 'Some nights I think it would be kinder if neither of us woke up.' The client mentions keeping a handgun in the bedroom. What should the social worker do **FIRST**?

A. Ask whether the thoughts include ending both their lives

B. Arrange respite care to ease the caregiving load

C. Counsel the client to store the handgun elsewhere

D. Explore the grief of watching the spouse decline

Why each option is right or wrong

A **Correct.** Assessment comes before intervention. 'Neither of us' signals possible thoughts of harming the spouse as well as oneself, and studies of spousal homicide-suicide in later life find that perpetrators are often caregivers with depressed mood (Malphurs & Cohen, 2005). Asking directly about intent toward both people shows how urgent and what kind of response is needed. It beats C because lethal-means counseling is essential and should follow quickly, but the plan depends on whether the risk includes the spouse.

B **Incorrect** (trap: Right action, wrong time). Respite may relieve the strain behind the statement and belongs in the plan. Arranging it before asking about intent leaves possible danger to the client and spouse unassessed.

C **Incorrect** (trap: Right action, wrong time). Removing access to the handgun is a vital step once risk is clarified, and it may come within minutes. First the social worker needs to know whether the thoughts involve the spouse, which shapes who must be protected.

D **Incorrect** (trap: Good but not best). Anticipatory grief is real and worth exploring later. Focusing on it now treats the statement as sadness alone and skips assessing a possible plan to harm two people.

Takeaway: When a caregiver's death wish includes the care recipient, assess thoughts of harming both before planning support or means safety.

Sources: Malphurs, J. E., & Cohen, D. (2005). A statewide case-control study of spousal homicide-suicide in older persons. American Journal of Geriatric Psychiatry, 13(3), 211-217, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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122. INT-1083 (rev 1) · Best answer: A

A program director at a child advocacy center learns that several interviewers and family advocates have come to see most strangers as dangerous, check on their own children many times a day, and describe the world as unsafe. The staff say they still believe in the work and want to stay. What is the **BEST** organizational response?

A. Build regular trauma-focused supervision into each staff member's week
B. Refer each affected staff member to the employee assistance program
C. Hold a staff wellness day with yoga and chair massage

Why each option is right or wrong

A **Correct.** Vicarious trauma is a cumulative change in a helper's inner beliefs about safety, trust and other people, caused by empathic engagement with clients' trauma (Pearlman & Saakvitne, 1995). Several staff now show these shifts, so the response belongs at the program level: regular supervision that names trauma exposure, along with caseload balance and ongoing self-care, which the Code calls paramount for competent practice (NASW Code, Purpose). It beats B because individual counseling can help, but referral treats a predictable job hazard as each person's private problem and leaves exposure unchanged.

B **Incorrect** (trap: Hands off too soon). Counseling through the EAP can help an individual, but several staff show the same changes from the same work. Handing each person off leaves the program's exposure and supports unchanged, so the effects are likely to recur.

C **Incorrect** (trap: Good but not best). A wellness day is a kind gesture and may ease stress briefly. It does not address the ongoing trauma exposure or the changed beliefs, which call for steady supervision and workload practices rather than a single event.

Takeaway: Vicarious trauma is an expected hazard of trauma work; address it through ongoing supervision, caseload balance and self-care built into the program.

Sources: Pearlman, L. A., & Saakvitne, K. W. (1995). Trauma and the Therapist: Countertransference and Vicarious Traumatization in Psychotherapy with Incest Survivors. W. W. Norton, addressing vicarious traumatization: supervision, balance and self-care; NASW Code of Ethics (2021), Purpose
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3